

NRCME Memory Palace — Exam Cheat Sheet

Compiled from public FMCSA sources: 49 CFR Parts 383, 390, 391, 395 and Appendix A to Part 391; the Medical Examiner's Handbook (2024 edition); the 2002 Cardiovascular Advisory Panel guidelines; and the Medical Review Board's 2016 sleep-apnea recommendations. Web edition generated September 16, 2026.

How to read: bold = tested number or phrase. Source tags: [reg] regulation text · [MEH24] 2024 Handbook · [legacy] pre-2024 FMCSA guidance · [clinical] clinical reference · [training] common training-course convention. Regulations (49 CFR 391.41–391.49) are binding; advisory criteria and Handbook guidance inform the examiner's judgment.

A Trident Alliance Holdings product. Not affiliated with or endorsed by FMCSA. Educational use by licensed clinicians; verify decisions against current regulations.

Numbers at a Glance

Source tags used throughout this guide: [reg] = 49 CFR text · [MEH24] = 2024 Medical Examiner's Handbook · [legacy] = pre-2024 FMCSA guidance · [clinical] = standard clinical reference, not FMCSA · [training] = common NRCME training-course convention (not FMCSA text).

The 13 standards — 49 CFR 391.41(b)

#	Standard	#	Standard
1	Loss of foot/leg/hand/arm (or SPE)	8	Epilepsy / LOC — objective standard
2	Limb impairment affecting prehension/power grasp (or SPE)	9	Mental / psychiatric disorder
3	Insulin-treated diabetes (must meet 391.46)	10	Vision — objective standard
4	Cardiovascular (MI, angina, coronary insufficiency, thrombosis; syncope/dyspnea/collapse/CHF)	11	Hearing — objective standard
5	Respiratory dysfunction	12	Drug use (Schedule I = never; II–V need prescription exception)
6	High blood pressure	13	Current clinical diagnosis of alcoholism
7	Rheumatic/arthritic/orthopedic/muscular/neuromuscular/vascular		

Hard thresholds

Item	Number
Vision acuity	20/40 each eye AND both eyes, corrected OK
Visual field	70° horizontal meridian, each eye
Colors	Red, green, amber
Hearing — whisper	5 ft , better ear, aid OK
Hearing — audiometry	Avg loss ≤ 40 dB at 500 / 1,000 / 2,000 Hz , better ear (ANSI Z24.5-1951)
BP Stage 1	140–159 / 90–99 → 1 yr
BP Stage 2	160–179 / 100–109 → one-time 3 months , then 1 yr if ≤ 140/90
BP Stage 3	≥ 180/110 → disqualify ; when ≤ 140/90 → 6 months, then every 6 months
Treated HTN, controlled	max 1 yr (never 2)
ETT "satisfactory"	> 6 METs (Bruce stage 2), HR ≥ 85% predicted, SBP rise ≥ 20, no angina/ST changes
LVEF	≥ 40% to certify (MI, CABG, HF) [legacy]; band labels (55–70 normal ... < 35 severely low) [clinical]
Asthma FEV1	no cert if < 65% predicted
Pulse ox / ABG	SpO2 < 92% → ABG; PaO2 < 65 mm Hg (< 5,000 ft) or PaCO2 > 45 → may not meet
Urine SG normal	1.005–1.030 [clinical]
AUDIT / CAGE	AUDIT ≥ 8 ; CAGE ≥ 2 [clinical — FMCSA mandates neither]
CBD	must be < 0.3% THC ; marijuana = Schedule I

Waiting periods

Heads-up: these come from the older Appendix A / Cardiovascular Advisory Panel tables that most NRCME training courses and practice banks still use. The 2024 ME Handbook dropped fixed waiting periods and 1-year cardiac intervals in favor of individual assessment ("up to 2 years"). Regulation-based numbers (HTN stages, seizures, ITDM, vision) are unchanged. See the last section.

Condition	Wait
MI	2 months (ETT at 4–6 wks; repeat every 2 yrs)

Condition	Wait
Angina (stable)	3 months free of rest angina / pattern change
PCI (elective, uncomplicated)	1 week per FMCSA's 2002 Cardiovascular Advisory Panel guidance; many training courses teach 3 weeks — be ready for either
CABG	3 months
Pacemaker	3 months (syncope) — remedial, not disqualifying
ICD	never certify
AF / SVT / ventricular arrhythmia (treated)	1 month
Valve repair/replacement, aneurysm repair, PVD surgery, PE	3 months
Neurocardiogenic syncope	3 months after treatment
Epilepsy	10 yrs seizure-free off meds (exemption: 8 yrs, meds stable 2 yrs → 1-yr cert)
Single unprovoked seizure	5 yrs off meds (exemption: 4 yrs → 2-yr cert)
Provoked seizure	none once resolved
LOC, unknown cause, not seizure	6 months
TIA / stroke (mild)	1 yr ; cortical/subcortical stroke 5 yrs
Mild TBI (no LOC)	1 month; moderate TBI 3 yrs ; severe (LOC ≥ 24 h) → do not certify
Bacterial meningitis w/ early seizure	5 yrs; viral encephalitis w/ early seizure 10 yrs
Benign CNS tumor post-op	1 yr (infratentorial); 2 yrs (supratentorial)
Vertigo (BPPV)	2 months symptom-free
Major depression (non-psychotic, no suicidality)	6 months symptom-free; severe / suicide attempt / mania → 1 yr
Schizophreniform / brief reactive psychosis	6 months; other psychoses 1 yr; schizophrenia dx → do not certify
ECT	6 months symptom-free, not on maintenance
Sedating med / antihistamine / narcotic antitussive	no driving for 12 h
ITDM, hypoglycemia	severe hypoglycemic episode (needs assistance / LOC / seizure / coma) → driving prohibited until the treating clinician confirms the cause is addressed, regimen is stable, and completes a new MCSA-5870 (391.46(e)); the form asks about the preceding 3 months — many training courses treat "an episode within the last 12 months" as disqualifying

Certification intervals

Situation	Max
Everything normal	2 yrs (never longer)
Insulin-treated DM (391.46)	12 months ; only 3 months if < 3 months of glucose records
Alternative Vision Standard (391.44)	1 yr
Hearing exemption, hearing aid	2 yrs
Seizure exemption	epilepsy 1 yr; single unprovoked 2 yrs
SPE certificate	2 yrs (SPE itself valid 2 yrs; renew 30 days before)
Most cardiac conditions	1 yr
Chronic venous disease, varicose veins, autonomic neuropathy	2 yrs
Periodic-monitoring boxes	3 mo / 6 mo / 1 yr / other

Forms & deadlines

Form	What	Deadline
MCSA-5875 MER	The exam record	keep 3 yrs
MCSA-5876 MEC	Certificate — driver original, employer copy on request	keep 3 yrs ; expires midnight, no grace
MCSA-5850	Results to National Registry	midnight, next calendar day

Form	What	Deadline
MCSA-5870	ITDM assessment by treating clinician	ME exam within 45 days
MCSA-5871	Vision evaluation (ophthalmologist/optometrist)	within 45 days
MCSA-5872	Non-insulin DM — optional	—
MCSA-5895	Driver medication form — optional	—
Determination pending	no MEC; driver returns	within 45 days or exam invalid
Records to FMCSA / law enforcement		within 48 hours
ME profile change		30 days ; NOPR ack 30 days, fix 60 days
ME refresher / recert		5 yrs / 10 yrs
Exam	120 Qs (reportedly ~100 scored), 2 h, 4 options [training]	pass 71%

CMV & CDL definitions

	Weight	Passengers	Hazmat
CMV (needs DOT physical)	≥ 10,001 lb GVWR/GCWR	> 8 for compensation; > 15 not for compensation (incl. driver)	placard quantity
CDL (drug testing, SDLA reporting)	≥ 26,001 lb	16+ incl. driver	placarded

Hours of service: 11 h driving / 14 h duty / 60 h per 7 days or 70 h per 8 days.

FMCSA and the Medical Examiner's Role

FMCSA / NRCME structure

- FMCSA = 1 of 9 DOT administrations; created by **Motor Carrier Safety Improvement Act of 1999**; HQ Washington, DC.
- FMCSA's **Medical Programs Division** administers the medical program: oversees the **National Registry (NRCME)** and the national certification process, the medical regulations, and the **Medical Review Board**; runs the **exemption and certificate programs**.
- FMCSA Large Truck and Bus Crash Facts (2017): 13% of fatal crashes involved a large truck/bus; ~4 of 5 deaths were occupants of other vehicles [FMCSA data]. **1999 Mother's Day crash** (22 dead): NTSB — driver incapacitation; driver had been certified despite disqualifying conditions.

Regulations vs. guidance

	Regulations	Guidance (e.g., Medical Advisory Criteria, Appendix A to Part 391; evidence reports; MEP opinions)
Force of law	Yes — mandatory	No — advisory, recommendations only
Key words	"must," "shall," "required"	"consider," "may," "should," "could"
ME may decline to use?	No	Yes — ME free to choose

FMCSA expects MEs to **individually assess each driver** against the regulatory standards; guidance may be considered; **ME makes the final determination**. Standards built from evidence reports, expert opinion, Medical Review Board; open to public comment.

ME certification and continued listing (49 CFR 390 Subpart D)

Eligibility: MD, DO, DC, APRN/NP, PA, or other professional authorized by state law to perform physicals; complete FMCSA-compliant training; register on National Registry (10-digit number); pass exam at FMCSA-approved testing org (passing **71%**).

Requirement	Number
Refresher training	Every 5 years (in 5th year of 10-yr cycle; free via NR profile)
Recertification (retake training + pass exam)	Every 10 years (in 9th or 10th year)
Report profile changes (address, phone, email, license expiration)	Within 30 days
Review NR profile	Periodic review (quarterly or better) is a [training] recommendation — the regulatory duty is the 30-day change report
Records/licensure/training certificates to FMCSA or federal/state/local law enforcement	Within 48 hours of request
Retain original MER (MCSA-5875) + copy of MEC (MCSA-5876)	At least 3 years (from exam/issue date)
Submit exam results (MCSA-5850) to National Registry	By midnight local time, next calendar day after exam
Months with zero exams	Report via NR account (by last day of month)
Minimum number of exams	None required
Reinstatement after removal	Wait 30 days to request; not guaranteed

Section map: 390.103 eligibility · 390.105 training · 390.107 testing · 390.109 credential · **390.111 continued listing** · 390.113 removal reasons · 390.115 removal procedure · 390.123–.135 VA examiners.

Audits: random, for cause, or eligibility (2024 enhanced monitoring). NOPR via NR account → **acknowledge within 30 days** → **complete corrective actions within 60 days** → confirm in account; failure = removal.

Key definitions (49 CFR 390.5 / 383.5)

Term	Definition
CMV (interstate)	GVWR/GCWR or GVW/GCW ≥10,001 lb (whichever greater); OR >8 passengers incl. driver for compensation ; OR >15 passengers incl. driver not for compensation ; OR hazmat in placard-required quantity
CDL vehicle	≥26,001 lb ; OR 16+ passengers incl. driver ; OR hazmat per 49 CFR 383.5. CDL data + med cert status kept by SDLA
Non-CDL driver	Operates 10,001+ lb / >8 pax for hire / >15 pax not for hire / placarded hazmat; not tracked by SDLA
Commercial driver	Anyone operating a CMV, CDL or not
Interstate commerce	Crosses state lines (incl. outside US), or intrastate leg where origin/final destination is outside the state/US
Intrastate	Entirely within one state, final destination in origin state, not meeting interstate definition
Medical examiner	Certified by FMCSA and listed on NRCME per 49 CFR 390

Term	Definition
Licensed medical practitioner	Per 391.41(b)(12): licensed to prescribe controlled substances, familiar with the driver's medical history and assigned duties
Treating clinician	Manages and prescribes insulin for diabetes
Treating provider	Treats/coordinates care of the driver (PCP or specialist)
MRO / SAP	MRO: physician reviewing employer drug-test results. SAP: evaluates DOT drug/alcohol violators, recommends treatment/follow-up. MEs do not perform drug tests (Omnibus Transportation Employee Testing Act of 1991)
Variance / exemption	Temporary relief from a standard, granted only by FMCSA — ME cannot issue

CFR citations that matter

Cite	Content
49 CFR 300–399	FMCSRs (Part 40 drug/alcohol testing; 100–180 hazmat; 571 vehicle safety)
390.3	Applicability to interstate commerce; employers may be stricter
391.41	13 physical qualification standards
391.43	ME responsibilities: exam instructions, driver tasks, advisory criteria, sample MER, MEC; MER mandatory as-is. (g)(2) MEC copy to employer on request; (g)(3) disqualified driver reported to FMCSA (eff. June 23, 2025); (i) records within 48 h
391.44	Alternative Vision Standard (replaced vision exemption); Vision Evaluation Report MCSA-5871 by ophthalmologist/optometrist, ME retains; certify at least annually
391.45	Who must be examined: never certified; not certified in preceding 2 yrs ; exempt intracity zone not certified in preceding 1 yr ; insulin-treated DM not certified in preceding 12 months ; ability impaired by disease/injury; fails vision with worse eye (annual); found not qualified under 391.43(g)(3)
391.46	Insulin-treated DM standard; treating clinician completes MCSA-5870 (to ME within 45 days); ME retains
391.47	Conflict/dispute resolution when the driver's medical examiner and the motor carrier's medical examiner disagree (391.47(b)(2))
391.49	SPE certificate program (fixed extremity deficit)
391.62	Exempt intracity zone exceptions
391.11 / 391.15	General driver requirements (age 21, English, road test) / disqualification
392.3 / 392.4	No driving ill/fatigued; no drugs/alcohol on duty
382, Part 40, 21 CFR 1308, 395	CDL drug testing; MRO/SAP procedures; 5 drug schedules; hours of service
Appendix A to Part 391	Medical Advisory Criteria (guidance)

13 standards — 391.41(b)

#	Standard	Discretion
1	No loss of foot/leg/hand/arm — or SPE certificate	D
2	No hand/finger impairment affecting prehension/power grasp; no limb impairment/defect interfering with driving — or SPE	D
3	No insulin-treated DM unless meets 391.46	D
4	No current dx of MI, angina, coronary insufficiency, thrombosis, or CV disease with syncope, dyspnea, collapse, CHF	D
5	No respiratory dysfunction likely to interfere	D
6	No high BP likely to interfere	D
7	No rheumatic/arthritis/orthopedic/muscular/neuromuscular/vascular disease that interferes	D
8	No epilepsy or condition likely to cause LOC/loss of control	Non-discretionary
9	No mental/nervous/organic/functional/psychiatric disorder that interferes	D
10	Vision: 20/40 each eye (corrected OK), binocular 20/40, 70° horizontal field each eye, recognize red/green/amber ; else 391.44	Non-discretionary
11	Hearing: forced whisper at ≥5 ft (aid OK) OR better-ear average loss ≤40 dB at 500/1,000/2,000 Hz (ANSI Z24.5-1951)	Non-discretionary
12	No Schedule I drug, amphetamine, narcotic, habit-forming drug; other scheduled drugs only if prescribed by licensed practitioner familiar with history who advises no effect on safe driving	D
13	No current clinical diagnosis of alcoholism	D

Non-discretionary = objective criteria; failing = **must not certify**. Other 10 = ME clinical judgment.

Exemption programs

Program	Status	ME action
Hearing exemption	Current (interstate only)	Certify "otherwise qualified," max 2 yrs ; mark "accompanied by" exemption, write "hearing"; do not also check "wearing hearing aid." Driver carries exemption + MEC
Seizure exemption	Current	Epilepsy dx: max 1 yr ; single unprovoked seizure: max 2 yrs ; mark "accompanied by federal seizure exemption"
Vision exemption	Gone — replaced by 391.44 Alternative Vision Standard	Annual cert; MCSA-5871 within 45 days
Diabetes exemption	Gone — replaced by 391.46	MCSA-5870; 12-month cert
Grandfathered programs	Expired	—
SPE certificate (391.49)	Current; not an exemption — alternative standard for fixed limb deficit	Physiatrist/orthopedist review → FMCSA issues; valid up to 2 yrs , renew 30 days before expiry; driver carries SPE + MEC

Waiver = state-specific (intrastate); exemption = federal, FMCSA-granted. ME determines only whether driver is "otherwise qualified."

Certification interval

Situation	Max period
Meets all standards	2 years (never longer, for any driver)
ME judgment / guidance	Shorter at ME discretion (MER "periodic monitoring": 3 mo, 6 mo, 1 yr, other)
Stage I HTN (140/90–159/99), first instance	1 year
Stage II HTN (160/100–179/109)	One-time 3-month cert; then up to 1 yr if ≤140/90 and treatment tolerated
Stage III HTN (≥180/110)	Disqualify until ≤140/90; then 6 months
Insulin-treated DM (391.46)	12 months
Alternative vision (391.44)	1 year
Exempt intracity zone	1 year
Determination pending	No MEC; up to 45 days to obtain info; return date ≤45 days; else exam invalid, re-examine; update via MCSA-5850

MEC expires **midnight** on expiration date, no grace period; certificate date = exam date.

MER determination options (federal section)

Does not meet standards (explain, discuss, report to FMCSA) · Meets standards, 2-yr cert · Meets standards, periodic monitoring required (reason + period) · Check boxes: wearing corrective lenses / wearing hearing aid / accompanied by waiver-exemption (specify) / driving within exempt intracity zone (391.62) / accompanied by SPE certificate · Determination pending · Incomplete exam. Never use "pending" for a condition that interferes with safe driving. [training] Second opinion after disqualification requires employer permission; second ME gets full history and is told it is a second opinion.

Who gets what

Document	Driver	Employer	SDLA	FMCSA/law enforcement
MEC (MCSA-5876)	Original (signed by both); carries it	Copy on request (391.43(g)(2)); carrier keeps copy	CDL/CLP: driver provides (paper) or electronic via NRII	Within 48 h
MER (MCSA-5875)	Copy on request — driver's right regardless of who paid (45 CFR 164.524)	Only with driver consent	Some states require copy	Within 48 h without consent

NRII rule (eff. June 23, 2025): MEC sent electronically NR → SDLA for CDL/CLP; 5 states not fully implemented (AK, CA, KY, LA, NH); FMCSA still recommends paper MECs; paper MEC valid as proof up to **60 days** after issue (through Oct 11, 2026). Non-CDL drivers always get paper. Intrastate results need **not** be uploaded.

ME role and obligations

- **No diagnosing, prescribing, or specific medical advice**; exam = public safety determination (interference with safe driving, or risk of incapacitation/sudden death).
- Do: thorough exam of all 13-standard systems; document on MER; consider meds/treatments; follow regs, consider guidance; request testing/specialist eval when unsure; **educate** on med side effects, labels, seeking care.

- Must disqualify: fails non-discretionary standard; public safety risk; permanent restriction interfering with driving. May disqualify: fails guidelines/advisory criteria, uncertain clinical course. Possible sudden incapacitation → get treating-provider info/refer **before** deciding.
- Consulting treating provider: **driver consent + release**; ask for actual records/results, not just recommendations; retain with MER; decision remains ME's.
- Intrastate-only drivers: follow **state** regs (may be stricter; know state waivers); **if in doubt, use federal**. Municipal and exempt intracity zone drivers may be exempt from federal standards unless state/employer requires. Domicile = SDLA's concern, not ME's.
- Motor carriers **may not influence** ME decisions or request MEC limitations.
- Anyone may receive a DOT physical; age 21, English, road test, single CDL = **motor carrier** responsibility (391.11).
- Certify for the most demanding CMV job, not the driver's current route.
- Office: no FMCSA facility mandate; Snellen at 20 ft; 5-ft quiet space for whisper; [training] driver changes into an exam gown for the physical.

Driver responsibilities

Comply with 391.41; recertify before MEC expires **or when health status changes**; use NR-listed ME (carrier may specify one); photo ID; accurate, complete, signed health history on MER (surgeries, all meds incl. OTC/supplements, 32 conditions, prior denial/short cert); supply treating-clinician info on request; CDL drivers give MEC (some states also MER) to SDLA — not required of non-CDL.

Privacy / PHI

- HIPAA (1996) Privacy Rule (HHS OCR); comply with federal + state law. Rule: **no PHI disclosure without HIPAA-compliant consent**.
- Without consent OK: serious and imminent threat (45 CFR 164.512(j)); MEC to current/prospective employer on request; records to FMCSA/federal/state/local law enforcement within 48 h (391.43(i)); complaint to FMCSA alleging "substantial violation" (49 CFR 386.12(a)).
- Employer who paid gets MEC on request, **not** the MER without consent.

Job of commercial driving

- Profile [training]: typically male, >40, sedentary, overweight, smoker, >2 medical conditions, CV disease prevalent; overall health poorer than the general population.
- HOS (49 CFR 395): **14-h** daily work, **11-h** max driving, **60 h/7 days** or **70 h/8 days**, log for last 7 days.
- Tasks [legacy]: mirror checks demand neck rotation; a large steering wheel requiring sustained grip and steering control; enter/exit with 3 points of contact, hip/knee flexion >90°; coupling (grip, upper body, ROM, balance); loading >6.0 METs; the regulations specify **no lifting weight**.

High-yield points

- The non-discretionary standards are **vision, hearing, and epilepsy** — the three with objective pass/fail criteria; HTN and diabetes are discretionary.
- Only two federal exemption programs remain: **seizure and hearing**. The ME never grants exemptions; grandfathered programs have ended.
- The MER is the **mandatory** exam documentation; retain **3 years**. It is not routinely transmitted anywhere — an employer sees it only with driver consent.
- MEC for CDL/CLP drivers is submitted to the **National Registry** (→ SDLA); non-CDL drivers receive paper. The original belongs to the driver; the ME keeps a copy 3 yrs; a MEC is issued only when the driver qualifies.
- Guidance is **advisory**; the ME may decline to follow it. FMCSA expects an **individual assessment of each driver against the regulatory standards**; consulting the advisory criteria, a specialist, or the treating provider is at the ME's discretion.
- Interstate vs intrastate turns on **crossing state lines / the shipment's origin and destination**; vehicle weight, hazmat, and passenger count define whether a vehicle is a CMV, not whether the operation is interstate.
- Federal rules touching drivers' health: medical certification, **drug/alcohol screening, hours of service**. Routine ECGs and screening audiograms are not federally required.
- The carrier verifies age ≥**21**, English proficiency, valid MEC, a single CDL, and the road test; the regulations set **no minimum lifting requirement**.
- Age is no bar to the exam: an under-21 applicant is **examined as usual** (age 21 is the carrier's hiring concern).
- Max certification **2 years**, never extended; shorter always allowed. Carrier-requested MEC limitations are prohibited.
- Attire [training]: driver gowned for the physical.
- Disqualifying by regulation: **current dx of alcoholism** (b)(13); **epilepsy** (b)(8). Most other chronic findings (elevated BP, past arthritis, non-insulin DM, obesity, myopia) call for individual assessment, not automatic disqualification.
- Insulin-treated DM **can** be certified when 391.46 is met — the pathway is the regulation itself, and it imposes no waiting period.
- ME duties include **health education/counseling**; diagnosis, prescribing, and treatment plans belong to treating providers. The exam screens for conditions creating an **unacceptable public safety risk**.
- Continued listing requires the 5-yr refresher, 10-yr recert, current license, and 30-day profile updates. There is **no minimum exam count** (zero-exam months are simply reported) and no annual CE-hour requirement.
- "Treating clinician" = **prescribes insulin**; "treating provider" = coordinates care.

- Intrastate-only driver → **know and follow state regs.**
- Records may go to **federal/state/local law enforcement** without consent (48-hour rule). The MER reaches an employer — even one who paid for the exam — only with driver consent, and exam results are not shared with a prospective employer or PCP without it. HIPAA still applies; FMCSA regs do not override it.
- CMV drivers as a population carry a **heavier illness burden** than average; typical driver is over 40; max **11 h** driving per shift.
- Cognitive demands of the job: problem solving, communication, judgment, appropriate behavior. Stressors: long relay routes, abrupt schedule changes, responsibility for passenger safety.
- The **ME alone** determines fitness for duty.

Vision & Hearing

VISION — 49 CFR 391.41(b)(10)

Non-discretionary standard. Three requirements, all must be met:

Requirement	Standard
Distant visual acuity	≥ 20/40 (Snellen) in each eye AND ≥ 20/40 binocular , with or without corrective lenses; record in Snellen-equivalent values
Field of vision	≥ 70° in the horizontal meridian of each eye
Color	Recognize colors of traffic signals/devices showing red, green, amber

- Fails any part, one or both eyes = not qualified. Near acuity NOT required.
- Color: recognizing standard red/green/amber meets standard even with a color deficiency.
- Testers: the ME, or an ophthalmologist/optometrist for the vision portion; [training] trained assistive personnel commonly perform the screening. ME makes final decision; signing MER attests to all results.

Corrective lenses / contacts / telescopic

- Lenses used in testing must be worn driving; mark "wearing corrective lenses" on **both** MER (MCSA-5875) and MEC. Uncorrected acuity need not be documented if standard met corrected.
- Contacts: verify tolerance/adaptation. Monovision (distance lens one eye, near lens other) acceptable if standard met.
- **Bioptic telescopic lenses: not permissible → disqualify.** Advise spare glasses/contacts.

Alternative Vision Standard — 49 CFR 391.44 (Jan 2022; replaced FMCSA exemption requirement)

- Monocular = worse eye fails the acuity standard **even with corrective lenses**, or fails the field standard (depth perception, contrast problems).
- **Better eye** must meet: ≥ 20/40 Snellen; ≥ 70° horizontal; red/green/amber.

Step	Detail
1. Specialist eval	Licensed ophthalmologist or optometrist determines deficiency is stable and enough time has passed to adapt/compensate; completes, signs, dates Vision Evaluation Report MCSA-5871
2. Form validity	45 days — driver brings it to ME within 45 days
3. ME exam	Driver must meet all other 391.41 standards; ME must consider MCSA-5871 info
4. Field testing (specialist)	Formal perimetry required; modality testing ≥ 120° horizontal; results in degrees, copy of perimetry attached to MCSA-5871
5. Certificate	Up to 12 months ; re-exam & recert every 12 months
6. Road test	Generally required for first-time AVS qualification; done after MEC issued and before interstate CMV operation; conducted by motor carrier per 49 CFR 391.31

Do NOT certify under AVS if: better eye < 20/40; better eye field < 70°; cannot recognize red/green/amber; deficiency not stable; insufficient time since stable.

When a driver fails vision screening

- Disqualify; instruct to see ophthalmologist/optometrist. **Do NOT use "Determination Pending" while the vision evaluation is being obtained.**
- If possibly AVS-eligible, have specialist complete MCSA-5871 → driver returns for a **new** physical qualification exam.
- "Determination Pending" only **after** driver has had the specialist eval and provided the completed MCSA-5871; gives up to **45 days** for clarification/additional info and to complete the exam.
- Inconclusive peripheral screen → refer to ophthalmologist/optometrist for precise field testing (may then be AVS candidate).

Specialist reports

- Required by regulation **only** for AVS. Report: name, phone, email, license no., state; signed/dated. Attach to MER, write "see attached documentation" in vision results.

Testing procedures

Test	Key details
Snellen	Chart 20 ft from driver (measure and mark); white light; numerator = 20 ft, denominator = last line read accurately; may skip to smaller lines; corrective lenses allowed; illiterate "E" chart OK; Titmus Vision Screener acceptable (acuity, depth, color, binocular)
Confrontation fields	Examiner and driver sit facing about 2 ft apart, eyes level; each covers the mirror-opposite eye and the driver fixates on the examiner's open eye; examiner extends arms and brings wiggling fingers in from the periphery, halfway between the two, testing above and below eye level; driver reports when movement is seen; repeat for the other eye. Finger-count variant along horizontal field, one eye closed at a time

Test	Key details
Color	Standard test (Ishihara, Pseudoisochromatic, Yarn, Farnsworth) or controlled test (standard colors on Snellen chart, or red/green/amber objects). Fails Ishihara → next step is a controlled test , not disqualification

History & physical

- Ask: vision changes, night vision, cataracts/glaucoma/retinopathy/macular degeneration, ophthalmic meds.
- Exam: pupils, nystagmus, exophthalmos, light reaction, accommodation, motility, EOMs, fundoscopy. Note on MER: ptosis, discharge, abnormal field, color blindness, corneal scarring, uncorrected strabismus.
- Some conditions fall under other standards (e.g., vascular disease, 391.41(b)(7)). ME not expected to diagnose → refer.

Conditions & meds

Condition	Exam-relevant points
Diabetic retinopathy	Insulin-treated DM + severe non-proliferative OR proliferative retinopathy = NOT qualified. Does not apply to non-insulin DM or retinopathy in general. Background retinopathy common after 5–7 yr DM. Affects contrast sensitivity, flicker fusion, color discrimination
Carcinoma-associated retinopathy	Photoreceptor degeneration → rapid onset blindness
Cataracts	Lens opacification; glare (night driving) early symptom; accelerated by injury, radiation, gout, steroids, DM; surgery + IOL; refer
Aphakia	No lens; hyperopia (lost accommodation); risk glaucoma, retinal detachment; not automatically disqualifying
Glaucoma	Abnormal IOP → optic nerve damage; painless; peripheral loss before symptoms ; Snellen may be normal; loss irreversible; meds need strict compliance, may have side effects. Normal IOP 10–20 mmHg [clinical]
Macular degeneration	Refer to ophthalmologist
Anisocoria	Uneven pupils; often benign but may signal nervous system disorder, viral infection, stroke risk
CN III palsy	Compression/ischemia; impaired eye movement, pupil reaction; diplopia in certain gazes, ptosis, dilated pupil → neurological evaluation
Nystagmus	Repetitive uncontrolled movements; reduced vision/depth perception, night vision; symptom of another problem; worse with fatigue/stress
Strabismus	Misalignment; diplopia, depth problems; note if uncorrected; not automatic DQ
Xanthelasma palpebrarum	Benign cholesterol plaques on eyelids; hyperlipidemia, thyroid dysfunction, DM
Xanthopsia	Yellow vision with digoxin
Floaters	May indicate retinal tear/detachment → defer pending ophthalmology eval
Ophthalmic preparations	Ask about side effects (stinging, burning, blurred vision, glare, reduced night vision, headaches, allergy); use may reveal disease (glaucoma). Check non-ophthalmic meds (antihistamines, anticholinergics)
Mydriatics	Dilated exam drops (hydroxyamphetamine, tropicamide) used by specialists for retinal exam

Vision certification intervals

Driver	Max interval
Meets 391.41(b)(10)	2 years
Alternative Vision Standard	1 year

HEARING — 49 CFR 391.41(b)(11)

Non-discretionary. Driver must EITHER: - First perceive a **forced whispered voice** in the **better ear** at not less than **5 feet**, with or without hearing aid; OR - Audiometry: average hearing loss in the **better ear** not greater than **40 dB** at **500, 1,000, 2,000 Hz**, with or without hearing aid (or cochlear implant); device calibrated to **ANSI Z24.5-1951**.

- Test **both ears**; only **one (better) ear** must pass.
- Give one test first, complete both ears. Pass → no second test. Fail → **must** give the other test (single failed test insufficient to disqualify). Fails whisper both ears → audiometry.
- Fails both tests → may see hearing specialist, then new physical qualification exam.

Forced whisper procedure

- Quiet area, **5-ft** distance; tested ear turned toward examiner, other ear covered; position out of the driver's view (no visual cues); **using the breath that remains after a normal expiration**, whisper words or random numbers (66, 18, 3); driver repeats. Avoid only sibilant words. Test both ears. Record distance (ft) first heard.

Audiometry

- Record dB loss at 500/1,000/2,000 Hz per ear; average = sum ÷ 3. Pass if better ear ≤ 40 dB. 3,000 Hz NOT required.
- ISO → ANSI conversion: subtract **14 dB** (500 Hz), **10 dB** (1,000 Hz), **8.5 dB** (2,000 Hz), then average.
- Worked example: R 28+34+36 = 98/3 = 32.7 pass; L 42+44+52 = 138/3 = 46 fail → **qualified** (one ear passes).
- Audiologist report (name, phone, email, license, state; signed/dated) attached to MER; "see attached documentation." [training] Assistive personnel may test; ME signs and is responsible.

Hearing aids

- Allowed for either test; office audiometers usually can't test with aids → audiologist.
- Driver says aid not needed: a driver who passes unaided takes no aid restriction; if the unaided test fails, retest with the aid.
- Qualifies with aid → must wear it (and keep it working) whenever driving; spare power source *encouraged* (2024 MEH) — [legacy] older guidance required carrying one, and many courses still teach "carry one." Cert up to **2 years**. No exemption needed.
- MER: the MCSA-5875 has only "Check if hearing aid used for test: Right Ear / Left Ear / Neither." MEC: check **"Wearing a hearing aid."**

Federal hearing exemption

Item	Detail
Who	Driver who does NOT meet standard even with hearing aids but meets all other 391.41(b)(1)–(13) standards
Scope	Interstate commerce only
Decision	FMCSA reviews the application and decides (apply: 202-366-4001, fmcsahearingexemptions@dot.gov, or online)
ME role	Determine whether standard met with aids (whisper with aids, or audiologist audiometry with aids); complete full physical
ME cert	Max 2 years
MER/MEC	Check "Accompanied by a waiver/exemption," write "hearing." Do NOT also check "Wearing a hearing aid" — never both
Driver carries	Current hearing exemption + current MEC
Responsibility for documentation	Motor carrier

Ear exam

Check for hearing aid, ear disorders, canal and TM abnormalities. Scarred TM, other ear 5 ft → certify 2 years.

Hearing intervals

Driver	Max interval
Meets standard (with or without aid)	2 years
Hearing exemption	2 years

High-yield points

- The field standard is **70°** per eye. A driver with a 62° field in one eye plus myopia/HTN → disqualify and refer (possible glaucoma).
- Acuity below standard disqualifies outright — there is no 3-month certificate or road-test workaround for vision. Corrected 20/40 R + 20/60 L = **disqualify** (each eye must reach 20/40); corrected 20/25 R + 20/70 L with binocular 20/40 = **disqualify**. Cataract with corrected acuity of 20/100 → disqualify and refer (no temporary cert).
- AVS: recert **every year**; MCSA-5871 within **45 days**; ME **must consider** the form. AVS is an alternative standard in the regulation, not an FMCSA exemption; the eye specialist — not the driver — signs the MCSA-5871.
- "Determination Pending" is correct only **after** the completed MCSA-5871 is in hand.
- Biopic telescopic lenses → disqualify regardless of a specialist's note.
- Contacts irritating but driver passes with them → certify wearing the contacts (a spare pair doesn't cure intolerance; the ME doesn't prescribe).
- Inconclusive confrontation field (e.g., after an orbital fracture) → refer for precise field testing first; AVS only if a deficiency is confirmed.
- Fails Ishihara → administer a controlled color test next.
- Self-limited surface conditions such as conjunctivitis need no specialist; retinopathy, cataract, glaucoma, and macular degeneration warrant referral.
- The one automatic regulatory eye disqualification: insulin-treated DM + severe non-proliferative/proliferative retinopathy (391.46). Aphakia, dry eyes, retinopathy in general, strabismus, subconjunctival hemorrhage, and light sensitivity call for individual assessment.
- Carcinoma-associated retinopathy can cause **rapidly progressive** blindness [clinical]. Glaucoma: painless, peripheral loss first, Snellen may stay normal [clinical].
- New floaters → defer pending ophthalmology.
- Hearing: whisper failed in both ears (e.g., 4 ft / 3 ft) → administer audiometry next; a driver may qualify by **either** test, so never disqualify on one failed test alone. Unaided 4 ft in one ear + aided 5 ft in the other = **passes** (better-ear rule).

- Audiometry: averages R 42.3 / L 44.0 → do not certify (may qualify if a repeat with an aid passes); 30+48+60 (avg 46) fails; R 32+40+42 (avg 38) passes → certify.
- Frequencies **500/1,000/2,000 Hz**; 3,000 Hz is not part of the standard; calibration is to ANSI Z24.5-1951 (ISO results get converted).
- Hearing aid drivers: cert **2 years**; whisper with the aid is acceptable; an audiologist is optional when office testing suffices; a cochlear implant needs no exemption.
- Exemption: FMCSA decides; motor carrier ensures documentation; MER "Accompanied by a waiver/exemption" + "hearing"; max 2 years.

Hypertension & Cardiovascular

Hypertension — 49 CFR 391.41(b)(6)

Standard: no *current clinical diagnosis of high BP likely to interfere* with safe CMV operation. HTN alone rarely causes sudden collapse; risk rises with **target-organ damage, esp. cerebrovascular disease**. Advisory criteria = recommendations, not mandatory (2002 CV Advisory Panel; adopted JNC-6, 1997). **39% of drivers with >20 yrs experience** have HTN; drivers more likely than other professions [training].

FMCSA staging / certification

Stage	BP	Certification
I	140/90 – 159/99	Low risk. 1 yr on first instance; recert annually, BP should be ≤140/90. At a <i>subsequent</i> exam, if >140/90 but <160/100 → one-time 3-month cert to reach ≤140/90
II	160/100 – 179/109	Absolute indication for drug therapy. One-time 3-month cert to start/adjust meds → then 1 yr if ≤140/90 and treatment tolerated
III	≥180/110	High risk of acute event. Do not certify, even briefly , until ≤140/90 and tolerated. Then 6 months, and every 6 months thereafter

- **Treated HTN:** max **1 yr** if ≤140/90 and meds tolerated (140/90 exactly = controlled → 1 yr). **Never 2 yrs** on antihypertensives.
- **Annual cert when ME does not know pre-treatment severity.**
- **Secondary HTN:** same stages. Evaluate for secondary cause if persistently hypertensive on max/near-max doses of **2–3 agents**; some causes fixable by surgery/specific drugs.
- Complications (ESRD, organ damage, PAD) → evaluate under the **applicable other standard** (PAD → (b)(4)).
- ME may always DQ when condition/prognosis uncertain (166/102 on meds + new headaches/dizziness → DQ pending treating-provider eval).

BP measurement / exam

- Use readings **taken during the exam**. Confirm any reading **>139/89** with a **second measurement later in the exam** (systolic or diastolic alone counts). The 2024 MEH states that readings greater than 139/89 *should* be confirmed with a second measurement later in the exam — guidance ('should'), not regulation. Fluctuation causes: circadian cycle, emotional/physical state, white-coat, left vs right arm, technique — any of these can move a reading.
- **Trained assistive personnel may measure/record BP; ME personally confirms any disqualifying reading** [training].
- Must measure **BP and pulse** (rate, strength, rhythm); record in MCSA-5875 Testing section, extra readings in "Second Reading" or abnormal-answer box. **Document all BP readings taken.**
- Advise: failure to reach ≤140/90 disqualifies; encourage PCP care.

Hypertensive emergency

Meningismus, sudden neuro deficits, abrupt SOB, tearing/ripping chest or back pain → stop driving, **immediate emergency care** (184/118 + SOB + dizziness = emergency, not just DQ). A driver at imminent risk of an acute hypertensive event is not certifiable until that risk is controlled.

Antihypertensives — side effects

- General: sleepiness, light-headedness, **syncope**, headache; evaluate individually; warn driver.
- Lisinopril: dizziness, headache, blurred vision. Enalapril + furosemide → ask about **HTN vs CHF history** (choose the right standard).
- Verapamil/CCBs: **bradycardia** (pulse 52 + fatigue → assess side effects; brady → dizziness, syncope, sudden death).
- Bisoprolol: most concerning = **dizziness/weakness**. Atorvastatin: drowsiness. Clopidogrel: **bleeding**.
- Sodium channel blockers: qualify under the *underlying condition's* standard — arrhythmia (b)(4), epilepsy (b)(8).
- Request provider info via **CMV Driver Medication form MCSA-5895**.

Cardiovascular — 49 CFR 391.41(b)(4)

Note: the waiting periods, "1 yr" intervals and ETT/LVEF cut-offs in the tables below are the older Appendix A / 2002 Cardiovascular Advisory Panel guidance still taught by most training courses. The 2024 ME Handbook lists these conditions as "up to 2 years" with individual assessment. Know both; see the final section.

Standard: no current clinical diagnosis of **MI, angina pectoris, coronary insufficiency, thrombosis**, or other CV disease **known to be accompanied by syncope, dyspnea, collapse, or congestive cardiac failure**.

Advisory criteria

- "Current clinical diagnosis" = current **or not fully stabilized**. "Known to be accompanied by" = accompanied by *or likely to cause* the 4 symptoms.
- **CABG and pacemaker = remedial** → **do not preclude certification**. ICD treats an ongoing condition; syncope/collapse from disease and discharge → **not certifiable**.
- **Anticoagulation alone never disqualifies**; judge the **underlying condition** and general health.

Exam principles

- Fitness includes **capacity for strenuous labor**; job tasks are deciding factors. Severe cerebrovascular disease → high MI/sudden-death rate.
- **Cardiologist recommendation is needed but not the decision.** ME applies the (b)(4) criteria and must weigh **actual test results, not clearance letters alone**; historical tables require "clearance from a CV specialist **who understands the job of commercial driving.**" Obtain records; attach tests to MCSA-5875.
- Undiagnosed findings (irregular rhythm at 132 bpm, S4/atrial gallop, exertional leg pain + fatigue, pulsatile abdominal mass) → **DQ/refer before certifying**, no interim cert.

Cardiac tests

- Ordered by ME, PCP, cardiologist, or CV surgeon; **interpreted by a cardiologist**; record in MER Section 6 or attach. ME may order ETT, echo, ECG — any.
- **ETT**: most common; workload in METs; **≥6 METs** = heavy work of commercial driving. Satisfactory: **>6 METs through Bruce stage 2, HR ≥85% predicted max (unless beta-blocked), SBP rise ≥20 mmHg without angina, no significant ST depression.**
- **Echo/LVEF**: **55–70% normal; 40–54% slightly low; 35–39% moderately low; <35% severely low.** Imaging beats plain ETT; use when resting ECG abnormal or ETT nondiagnostic.
- **Post-MI ETT at 4–6 weeks; repeat at least every 2 years** (source ties biennial ETT to CHD/post-MI follow-up; no age-45 rule in these materials).

Coronary heart disease

LV function = major independent predictor of survival; also general health, age, arrhythmias, angina, vascular disease, severity. Ask: tx effective/safe/stable? knows meds? compliant?

Condition	Wait	Certify if	Max	Do not certify if
Acute MI	min 2 months	asymptomatic; tolerates meds; satisfactory ETT ; resting LVEF ≥40% (in-hospital post-MI echo ≥40% suffices initially); no ischemic ECG	1 yr	rest angina/pattern change within 3 months ; ischemic rest ECG; intolerant of therapy
Angina	≥3 months free of rest angina/pattern change	stable, asymptomatic, tolerates meds, satisfactory ETT	1 yr	unstable angina within 3 months
PCI	1 week after elective uncomplicated PCI for stable angina per FMCSA's 2002 Cardiovascular Advisory Panel (longer post-MI/unstable); many training courses teach 3 weeks — be ready for either	asymptomatic; tolerates meds; no access-site injury (heals ~1 wk); cardiologist clearance	1 yr	access-site complications; rest angina; ischemic ECG
CABG	≥3 months any type	asymptomatic; no orthostatic sx; LVEF ≥40% ; healed sternum ; cardiologist approval	1 yr	symptomatic; orthostatic sx; LVEF <40%; unhealed sternum; no cardiologist eval
Heart failure	none (etiology confirmed, tx stable)	asymptomatic; tolerates meds; ETT >6 METs ; LVEF ≥40% , stable; compliant	1 yr	symptomatic HF regardless of EF ; asymptomatic LVEF <40% ; asymptomatic LVEF ≤50% + ventricular arrhythmias (VT/NSVT/palpitations)

- Stable angina triggers: exertion, emotion, weather extremes, sex. Unstable: rest pain, ↑frequency/duration, ↓med response → precursor to syncope/collapse/CHF.
- Post-MI: highest mortality first months (mostly sudden death); return to work if no exercise ischemia or LV dysfunction.
- CABG: for multivessel CHD, proximal left main, extensive disease with LV dysfunction/debilitating angina. Sternal risk first **3 months**; graft re-occlusion typically **after 5 yrs**; ETT *may* be indicated (not required); most drivers return to work.

Arrhythmias & devices

Ask: underlying disease disqualifying? sudden-death risk? cerebral hypoperfusion/LOC risk? Most sudden cardiac deaths = VT/VF, usually no prior dx; recurrence high even after defibrillation.

Condition	Wait	Certify if	Max	Do not certify if
ICD / ICD-pacemaker combo	—	Never (even asymptomatic with cardiologist clearance). Disabled ICD: only if underlying condition resolved, case-by-case	—	any ICD
Pacemaker (bradycardia: sinus node dysfunction, AV block)	remedial; syncope table: 3 months post-implant	no malfunction (syncope, persistent slow HR, fatigue, brady-tachy alternation); tx stable	1 yr	malfunction; disqualifying underlying disease

Condition	Wait	Certify if	Max	Do not certify if
SVT / AF	1 month (AF anticoagulated for stroke/risk/post-thoracic surgery; post-isthmus ablation; asymptomatic AVNRT, AVRT/WPW, atrial/junctional tach)	HR controlled; anti-embolic tx effective/tolerated; no underlying heart disease; CV specialist clearance	1 yr	LOC, compromised cerebral function, sudden-death resuscitation
Ventricular arrhythmia	1 month after therapy (CHD, RVOT VT, idiopathic LV VT)	asymptomatic; identified non-disqualifying cause; specialist clearance	1 yr	symptomatic; sustained VT ≥30 s needing intervention ; NSVT or LVEF <0.40 ; HCM; cardiac arrest; non-spontaneous sustained VT; abnormal exercise BP; family hx premature death; syncope
Autonomic neuropathy / POTS	none	etiology confirmed; cardiologist-confirmed stable tx	2 yrs	resting tachycardia or orthostatic BP; other-organ neuropathy impairing driving

- AF: risk = embolus → stroke/syncope/collapse; anticoagulation lowers embolization. Decision hinges on **whether underlying heart disease is disqualifying**. New AF on warfarin, no INR results → **DQ pending INR**.

Syncope

Symptom, not diagnosis. **Cardiac syncope** → **CV standard**; **neuro causes (migraine, seizure)** → **their own standards**. Ask: pre-syncope vs true LOC? meds causing electrolyte shifts? cause? treated? tx stable?

Type	Wait	Certify if	Max
Cardiac-based (incl. vasovagal)	none	asymptomatic; etiology identified and ≥1 month on treatment, or ≥3 months post-pacemaker	1 yr
Neurocardiogenic	3 months after treatment	symptom relief, asymptomatic (pacemaker fixes only the cardioinhibitory, not vasodepressor, component)	1 yr; DQ if symptomatic

Valvular disease

- Murmur grades I–VI (III = easily heard; IV = + thrill; V = partial stethoscope contact; VI = off skin). **Pathologic** → **evaluate**: holosystolic, harsh, abnormal heart sound, early/mid-systolic click, **grade ≥III**, left sternal border.
- Mechanical valves: no rejection, durable, **need anticoagulation**. Biologic: may be rejected, last **7–10 yrs**, usually no long-term anticoagulation.

Condition	Wait	Certify if	Max	Do not certify if
AR mild–moderate	none	asymptomatic; moderate needs normal LV function, little/no enlargement	1 yr	symptoms; moderate + abnormal LV; >mild LV enlargement
AR severe	3 months after AV repair	no sx; normal LV; LVEDD ≤60 / LVESD ≤50 mm	6 months unrepaired; 1 yr repaired	symptoms; <6 METs ; EF <50% ; LVEDD >70 / LVESD >55 mm
Aortic stenosis	3 months if repaired	mild or moderate asymptomatic; severe only if repaired	1 yr	moderate + angina/HF/AF/EF <50% /thromboembolism; severe AS regardless of sx/LV function
AV repair / prosthetic valve	3 months	LVEF >40% ; anticoag compliant; specialist clearance	1 yr	persistent sx; EF <40% ; thromboembolism; AF; pulmonary HTN; inadequate INR (checks ≥monthly)
Mitral regurgitation	3 months post-repair/commissurotomy	asymptomatic mild–moderate with normal LV & PA pressure; asymptomatic severe; or repaired + clearance	1 yr	symptoms; <6 METs ; ruptured chordae/flail leaflet; AF; LV dysfunction; thromboembolism; pulmonary HTN
Mitral stenosis	4 wks post-balloon valvotomy; 3 months post-surgical commissurotomy	mild/moderate asymptomatic; severe only after treatment + clearance	1 yr	severe MS until treated ; thromboembolism; pulmonary HTN
MVP	none	ME judges not dangerous	1 yr	symptoms/reduced effort tolerance; flail/ruptured chordae; emboli; AF; syncope or VT; severe MR/LV dysfunction
Pulmonary stenosis	1 month post-balloon; 3 months post-surgical	mild/moderate or corrected	1 yr	dyspnea/palpitations/syncope; peak gradient >50 mmHg ; RV pressure >50% systemic; >mild RVH/RV dysfunction; >moderate PR; main PA >5 cm

- **Mitral stenosis: most important = symptom development** (angina, syncope, fatigue, dyspnea). MR (most common valve disease): dyspnea, fatigue, orthopnea, PND = poor prognosis. AR: severity + LV size/function + symptoms.

Aneurysm, PVD, venous disease

Condition	Wait	Certify if	Max	Do not certify if
Aneurysm post-repair	3 months	asymptomatic; follow-up compliant; specialist clearance; anticoag monitoring	1 yr	symptomatic; repair recommended but not done
Acute DVT	none (etiology confirmed, tx stable)	no residual acute DVT; anticoag monitoring	1 yr	ineffectively treated
Chronic thrombotic venous disease	none	asymptomatic	2 yrs	ME judges dangerous
Claudication / PVD	3 months post-surgical repair	asymptomatic; no other disqualifying CV condition	1 yr	pain at rest
Pulmonary embolism	3 months PE-free	appropriate long-term treatment; anticoag monitoring	1 yr	symptoms
Superficial phlebitis	none	otherwise qualified	1 yr	coexisting DVT not meeting DVT rules
Varicose veins	none	no complications	2 yrs	venous insufficiency/ulcers/recurrent DVT

- **AAA:** rupture risk tied to **size** (ME's primary concern). Men 3:1, 60s–70s; **smoking strongest risk factor**; family hx, Caucasian. Exam detects ~90% of aneurysms >6 cm; bruit; ultrasound ~100% sensitive/specific for monitoring. **Thoracic asymptomatic <5 cm** unlikely to rupture. Pulsatile mass → **DQ pending CV consult, CT/US, specialist recommendations**. (No 4–5 cm tiers in these materials.)
- **DVT:** drivers at higher risk (sitting); anticoagulation cuts recurrence ~80%. Fresh DVT on warfarin without records → DQ pending treating-physician consult. Remote DVT clearance factors: symptoms, proximal extension, muscular-vein thrombus, D-dimer, ultrasound, bleeding risk.
- Claudication (7–9% of PVD): decide on **etiology confirmed + tx effective/safe/stable**, not the surgeon's note.

Anticoagulation

Historical: wait **1 month** after stabilization; max 1 yr; **INR results at exam, INR monitoring ≥monthly**; DQ if INR unmonitored/non-therapeutic or underlying disease disqualifying. Decision = **underlying disorder**, not the drug. (DOACs not addressed in these materials.)

Other conditions

Condition	Guidance
Heart transplant	Concerns: rejection, post-transplant atherosclerosis, med effects. Historical: wait 6 months , cert 6 months , NYHA I–II, LVEF >40% , no rejection, specialist clearance. Practical rule: evaluate individually
HCM	Case-by-case ; consider cardiology. Historical certify: no arrest, no VT/NSVT, no family hx premature sudden death, no syncope, septum <30 mm , clearance; DQ if peak gradient ≥50 . Sudden death may be first sign
Restrictive CM	Least common; stiff myocardium, impaired filling; arrhythmias common. Case-by-case
Congenital HD	HF and sudden death = major killers. Ask anatomy, severity, repair, cardiomegaly, sx, adult-CHD cardiologist. Ebstein: 1 yr if mild/asymptomatic; DQ if moderate–severe, intracardiac lesion, shunt, symptomatic arrhythmia, accessory pathway
Brugada/long QT, myocarditis/pericarditis, hypotension	Not addressed in these materials

High-yield points

- **≥180/110 → do not certify, even briefly** (184/112 — or 178/112, disqualifying on the diastolic alone → no certificate; advise prompt care). Once ≤140/90 → **6 months, then every 6 months**.
- **First encounter at Stage 2 (168/94, or 162/88 on the systolic alone) → one-time 3-month cert**; the ME certifies for 3 months rather than disqualifying, and prescribing is the treating provider's job.
- **Stage 1 untreated (150/94, 144/92) → 1 yr**; Stage 1 = *low* risk.
- **On meds, 140/90 × 2 → 1 yr**; treated HTN **never 2 yrs**.
- Returning driver >140/90 but <160/100 (e.g., 156/84 — the systolic alone counts) → **one-time 3-month cert**.
- Confirm any reading **>139/89** with a second measurement; assistants may measure, **ME confirms disqualifying values** [training]; document **all readings**.
- 184/118 + SOB/dizziness → **immediate emergency care**.
- Medication side effects worth knowing: antihypertensives as a class → **syncope**; verapamil → **bradycardia**; bisoprolol → **dizziness/weakness**; atorvastatin → drowsiness [course — clinically dubious]; clopidogrel → bleeding; request prescriber input via **MCSA-5895**.
- **ICD or ICD-pacemaker combo → not certifiable**, even with cardiologist clearance. **CABG, pacemaker = remedial**.

- **Anticoagulation decision = underlying disease.** AF on warfarin → **INR results first.** Asymptomatic AF → is underlying disease disqualifying?
- Newly found irregular rhythm at 132 bpm → **disqualify and refer**; no interim certificate.
- Post-MI 3 months, satisfactory ETT, EF 48%, cleared → **1 yr.** MI 5 wks, EF 52%, no ETT yet → **request the ETT** (4–6 wks post-MI; repeat q2 yrs).
- Stable angina 1 yr with ETT + clearance → **1 yr**; inconclusive ETT + recent exertional pain → **DQ pending testing.**
- PCI 3 wks → **cardiologist clearance**, check access site, compliance.
- CHD with NSVT, borderline METs, EF 36% → weigh LVEF stability, med compliance, and symptoms together.
- HCM, restrictive CM, transplant, pulmonary stenosis → **case-by-case / nature & severity.**
- Mitral stenosis → watch **symptom development**; AR → weigh severity, LV size/function, and symptoms together.
- S4 gallop → **cardiology referral.**
- AAA → **size**; pulsatile mass → DQ pending consult/imaging.
- Sodium channel blocker → **most applicable standard.**
- **6 METs**; tests **interpreted by cardiologist**; ME relies on **results, not clearance letters.**

Respiratory, Neuromuscular & Limbs

RESPIRATORY — 49 CFR 391.41(b)(5)

Standard: no established medical history or clinical diagnosis of respiratory dysfunction *likely to interfere* with safe CMV operation. **Advisory criteria:** (1) O₂-exchange conditions — emphysema, chronic asthma, carcinoma, TB, chronic bronchitis, OSA; (2) suspected undiagnosed/inadequately treated dysfunction → confer with treating provider or refer to specialist. **ME's primary goal:** establish whether disease increases risk of **sudden death or incapacitation**.

Exam findings → **pulmonary eval/consult:** clubbing, cyanosis, prolonged expiration, tachypnea at rest, wheezes/rhonchi/rales, absent/decreased breath sounds, pleural rub, unequal R/L thorax inflation, **significant thoracic kyphosis/scoliosis**, accessory muscle use at rest.

PFTs and thresholds

Test	Indicated when	Threshold
PFTs	Lung disease history; SOB/cough/chest tightness/wheeze; cigarette smoker ≥35 y	—
Spirometry (FEV ₁ , FVC)	Lung disease history or symptoms (not cor pulmonale, allergic rhinitis, or suspected OSA)	Asthma: no cert if FEV₁ < 65% ; abnormal → further eval before cert
Pulse oximetry	Condition causing airway obstruction	Normal 95–100%; request ABG if < 92%
ABG	Airway obstruction	May not meet standard: PaO₂ < 65 mm Hg (< 5,000 ft), < 50 mm Hg (> 5,000 ft), or PaCO₂ > 45 mm Hg

Oxygen therapy: supplemental O₂ needed for exertion → **disqualify** (even with pulmonologist note). Hypoxemia-at-rest clue: capillary refill > 2 s [training].

Conditions (Appendix A)

Condition	Wait	Max cert	Do not certify if / notes
Allergic rhinitis	None*	2 yr	Severe conjunctivitis affecting vision, can't keep eyes open, photophobia, uncontrollable sneezing, sinusitis w/ severe HA, sedating meds. Rarely disqualifying; use non-sedating antihistamine/nasal steroid
Allergy life-threatening (anaphylaxis, angioedema)	—	2 yr	No effective treatment AND no successful preventive measures. Suggest epinephrine device in vehicle
Asthma	None*	2 yr	Continual uncontrolled symptomatic asthma; FEV₁ < 65%; hypoxemia < 65 mm Hg . Case-by-case
Hypersensitivity pneumonitis	None	2 yr	Does not preclude; needs care; avoid causative agent
COPD	None*	2 yr	Hypoxia at rest, chronic respiratory failure, cough syncope . Smoking = primary etiology; insidious; cough, sputum, DOE
Acute infection (cold, flu, bronchitis)	None	2 yr	Relieve from duty until treated; no driving ≥12 h after sedating med ; not while contagious
Narcotic antitussives	Abstain 12 h before driving	—	Not if treatment interferes with driving
Chest wall deformity (kyphosis, pectus, AS, massive obesity, recent thoracic surgery)	None*	2 yr	Hypoxia at rest, chronic resp failure, cough syncope. Key = airway function ; sensitive to alcohol/antidepressants/sleep meds
Cystic fibrosis	None*	2 yr max; 1 yr recommended	Hypoxemia at rest, resp failure, cough syncope, spirometry not met, unstable regimen. Key = extent of disease + ability to obtain therapy while working ; specialist surveillance
Interstitial lung disease	None*	2 yr	Hypoxemia at rest, resp failure, cough syncope. PFTs if breathless driving/walking/stairs/cargo/entering cab
Pneumothorax	None*; x-ray must confirm resolution	2 yr	≥2 spontaneous on one side w/o successful surgical prevention ; hypoxemia at rest; resp failure; cough syncope. OK if asymptomatic, no disqualifying lung disease, single resolved or successful pleurodesis
Cor pulmonale	None*	2 yr max; 1 yr recommended	Dyspnea at rest, dizziness, hypotension, PaO₂ < 65 mm Hg ; vasodilator side effects
Pulmonary hypertension	None*	1 yr	Dyspnea at rest, dizziness, hypotension, PaO₂ < 65 mm Hg
Pulmonary TB	—	—	History does not preclude if etiology/treatment confirmed, compliant, infection resolved. Recurrence after treatment very low; incidence ~2.8/100,000 [course — dated stat]

Condition	Wait	Max cert	Do not certify if / notes
Atypical TB (M. avium-intracellulare)	—	—	Noninfectious; generally untreated; if on meds assess side effects
Primary idiopathic alveolar hypoventilation	—	—	Unlikely to meet standard

*Certify only after etiology confirmed and treatment proven effective, safe, stable. Lung transplant not addressed in sources.

Medications

- 1st-gen antihistamines (diphenhydramine): sedating up to **12 h** → defer cert pending PCP consult for non-sedating alternative. 2nd-gen (fexofenadine): most don't interfere.
- Montelukast concern: **ear pain/trouble hearing** → hearing standard (b)(11) [training]. Clinically, montelukast's headline warning is neuropsychiatric (FDA boxed warning).
- Hydrocodone (Vicodin) for cough: Schedule II, half-life 4 h, needs (b)(12)(ii) prescription exception → request prescriber info (MCSA-5895).
- Pirbuterol/albuterol listed → ask why (asthma?). Chronic cough for months on OTC meds → **refer to specialist**.

Obstructive Sleep Apnea (replaces Jan 2015 Bulletin)

- Regulations do **not** require screening, referral for testing, specific diagnostic/treatment methods, **compliance assessment**, waiting periods, or max cert periods. Further guidance: **Nov 21, 2016 advisory recommendations** (MRB Task 16-01). No CPAP hours/% nights criteria in sources.
- Treated moderate-to-severe OSA does **not** preclude. Key: treatment **effective, safe, stable**.
- FMCSA favors **multiple risk factors** (not one criterion): small airway history, loud snoring, witnessed apneas, sleepiness during major wake periods, obesity/high BMI, large neck, HTN, CVD, stroke/diabetes/comorbidities. Multiple factors → consider sleep study/board-certified sleep specialist unless previously evaluated. Prior negative/mild study → no repeat unless significant change. No routine retesting; treating provider decides.
- AHI**: apnea = no airflow > 10 s; hypopnea = ≥30% flow drop ≥10 s + O₂ drop > 3–4%. Mild ≥5/h, moderate ≥15/h, severe >30/h.
- Optional screening (AASM-style screen commonly taught in training courses)**: history — HTN needing >2 meds, T2DM, AF/nocturnal dysrhythmia, CHF, stroke, pulm HTN, drowsy MVAs, bariatric candidate. Findings — high Berlin/Epworth/STOP-BANG score, **neck > 17 in men / > 16 in women, Mallampati 3–4, retrognathia, macroglossia, large uvula, narrow palate, nasal abnormalities, BMI > 35**.
- Actual Nov 2016 FMCSA MRB (Task 16-01) recommendation**: refer for sleep study at **BMI ≥ 40**, or **BMI 33–40 plus ≥3 risk factors**; neck > 17 in (male) / > 15.5 in (female).

NEUROMUSCULAR — 49 CFR 391.41(b)(7)

Standard: no established history/diagnosis of **rheumatic, arthritic, orthopedic, muscular, neuromuscular, or vascular** disease interfering with safe operation. **Advisory criteria**: diagnosis = established history; consider nature/severity, degree of limitation (ROM), rate/stage of progression, interference; **severe functional impairment = not qualified**; may certify < max period for monitoring.

- Certification covers driving AND non-driving/heavy labor, every employer and vehicle type. **ME may not add restrictions** (e.g., lifting limits) — employer's discretion; unable to do normal tasks → disqualify.
- Job demands: unload up to **50,000 lb** after prolonged sitting; tire chains **35–90 lb**; tarp **50–100 lb**; steering needs power grasp + prehension; gear shift = right arm/hand + left lower leg; backing needs depth perception. Driving is the *least* demanding part.
- Exam: equilibrium/coordination, speech, DTRs, patellar/Babinski, sensory/positional, ataxia, alertness/comprehension (alcohol/drug history not part of this portion). **No test required** — grip test (dynamometer/sphygmomanometer) optional; specialist referral at ME discretion.

Category	Examples / notes
Episodic	Acute episodes (weakness, ataxia, visual); worse and longer each recurrence
Progressive	Insidious, months–years; weakness, rigidity, loss of control, numbness, twitching, cramping, joint deformity; interfere only late
Abnormal muscle excitability	Myotonia, Isaac's, stiff-person — most likely preclude

Condition	Determination
Multiple sclerosis	Case-by-case; no specialist clearance mandated. Lhermitte's sign, optic sx, relapsing-remitting. "Possible MS," stable, 4/5 arm, normal grip → certify 1 yr . Not SPE-eligible (progressive). Future disqualifiers: progression, excessive fatigability (heat/stress), functionally significant signs, new lesions ≥1 yr apart
Parkinson's	Case-by-case; advanced: orthostatic hypotension, cognitive decline. Sinemet concern: dizziness , daytime sleepiness
Peripheral neuropathy; muscular dystrophy	Case-by-case
Autonomic neuropathy	Max 2 yr ; not if resting tachycardia/orthostatic BP or other organ interference

Condition	Determination
ALS, myasthenia gravis, metabolic muscle disease (PKU, homocystinuria, MSUD)	Disqualifying per guidelines
Epilepsy / narcolepsy	Epilepsy disqualifying by regulation (only always-disqualifying episodic condition); narcolepsy disqualifying by guidance; headaches usually not
Lewy body dementia	Progressive, untreatable; impairs driving
Carpal tunnel	Test grip/prehension first; weak grip → likely disqualify
Arthritis / back pain	Case-by-case, weigh physical tasks. Can't climb or flex knees > 90° → disqualify . Back pain w/ normal strength/ROM → best: refer to PCP (could be 2 yr; no lifting limits). Duloxetine: drowsiness. Methocarbamol: dizziness/drowsiness/syncope, half-life 1–2 h, 10 h to clear

Neurology waiting periods (Appendix A)

Condition	Wait	Max cert	Notes
TIA	1 yr	1 yr	Normal neuro, neuro-optho, neuropsych evals; no residuals; neurologist clearance. Not if oral anticoagulant or CNS depressant
Embolic/thrombotic stroke	1 yr if mild, no seizure risk; 5 yr cortical/subcortical	1 yr	Same
ICH/SAH	1 yr no seizure risk; 5 yr if seizure risk	1 yr	Same
Mild TBI (LOC < 60 min)	1 month if no LOC ; symptom-free per treating MD/DO; LOC → neurologist	—	No HA, dizziness, imbalance, fatigue, personality/cognitive change, seizures, intracranial blood
Moderate TBI (LOC 30 min–24 h)	3 yr	6 mo in active treatment; then annual × 3 yr; then ME discretion	MD/DO clearance + neurologist (motor speed, dexterity, cognition, depression). Seizure → (b)(8). (Note: the legacy mild [<60 min LOC] and moderate [30 min–24 h LOC] definitions overlap — an inconsistency in the old FMCSA tables)
Severe TBI (LOC ≥ 24 h) / penetrating	—	—	Do not certify
Vertigo (BPPV, peripheral vestibulopathy)	2 months symptom-free	2 yr (annual if deficit needs special screening)	Never: Meniere's, labyrinthine fistula, nonfunctioning labyrinths
Headaches	None*	2 yr	Etiology confirmed, stable
CNS infection	Meningitis/encephalitis no early seizure: 1 yr; bacterial meningitis + early seizure: 5 yr; viral encephalitis + early seizure: 10 yr (seizure-free, off meds)	2 yr	Aseptic meningitis OK; active infection no
CNS tumors	1 yr seizure-free post-op : benign infratentorial meningioma, acoustic neuroma, pituitary adenoma, spinal benign, extra-axial. 2 yr : benign supratentorial, spinal	1 yr	Not recommended: primary/metastatic malignancy, benign nervous system tumors
Seizure	Provoked: none; <i>unprovoked</i> : 5 yr* off meds	1 yr	Epileptologist clearance

Spinal cord injury, joint replacement, fusion, amputation, ortho surgery: no waiting periods in sources — case-by-case; fixed limb deficit → SPE route; spine/torso → no SPE.

LIMBS — 391.41(b)(1), (b)(2), 391.49

Rule	Text	Key
(b)(1) Loss	No loss of foot, leg, hand, arm — OR SPE	Hand loss = all 5 fingers. Loss precludes without SPE even with prosthesis. Visualize all extremities
(b)(2) Impairment	No hand/finger impairment interfering with prehension or power grasp ; no arm/foot/leg or other limb defect interfering with normal tasks — OR SPE	< 5 fingers or any toes → (b)(2) functional eval. SPE unavailable for a spine/torso impairment <i>that does not result in a limb impairment</i> (391.49); a spine problem causing a fixed limb deficit (e.g., spinal stenosis w/ foot drop) is evaluated on the limb
391.49	Fails (b)(1)/(b)(2) but otherwise qualified → may drive if FMCSA grants SPE	Eligible under (b)(1), (b)(2), or both

SPE facts

- **Fixed** deficit only (stable over lifetime); not progressive disease (MS). Fixed status judged by board-qualified/certified **physiatrist or orthopedic surgeon**; **FMCSA** makes final decision and issues (only FMCSA grants SPEs/exemptions).
- FMCSA may require demonstration to an agent: pre-trip/non-driving, off-highway, on-highway — not always required.
- **Valid up to 2 yr**; renew **30 days** before expiration; may restrict prosthetic/equipment use. Driver **carries SPE + MEC** while driving.
- Hand/arm loss: prosthesis must allow **precision prehension** (knobs/switches, thumb+fingers) and **power grasp prehension** (steering wheel, whole hand). Current 391.49: demonstrate both **with each hand separately**; if not, fitted with prosthesis/orthotic **before applying** (post-Jan 6, 1986 drivers).
- Do not certify: impairment of **torso**; no proof of SPE compliance; limb impairment from progressive disease.

ME role

1. SPE presented → **proceed with exam**; evaluate unaffected limbs normally.
2. Address "Yes"/"Not sure" answers; answer **question 19**; mark **question 11 (extremities/joints) Abnormal**; document location/extent, how/when, prosthesis, interference — specific enough for FMCSA.
3. Determination: "**Meets Standards**" + "**Accompanied by SPE certificate**" on MCSA-5875 and MCSA-5876 (federal option; complete federal OR state section only).
4. Give driver MER + MEC for application. PCP clearance **not** required.
5. **Max cert with SPE = 2 yr** (e.g., BKA + SPE → 2 yr). The federal seizure exemption is the one program keyed to annual certification [course — hearing-exemption terms also involve periodic recertification].

Musculoskeletal exam

- Neck ROM (mirrors/backing), spine, extremities (brace, atrophy, deformity, leg length), gait, pedal strength, shoulder/elbow, precision + power grasp. **No required ROM values or grip test**. Ask about **previous spine/musculoskeletal surgery**, muscle-relaxant use, torso pain.
- Optional: railing grab, steering/gear simulation, lift known weights, one-leg balance, cab entry, box-and-block/nine-hole peg.
- Wrist fusion, grip 12 lb R vs 65 lb L → **disqualify**.

High-yield points

- COPD needing supplemental O2 for exertion → **disqualify**, even with a pulmonologist's note.
- Active asthma flare — wheezing, rescue inhaler several times a day → **disqualify and refer for pulmonary care**; there is no short-term certificate for an uncontrolled flare.
- COPD smoker, productive cough, SOB on exam → **disqualify pending pulmonary evaluation**; the referral comes before ordering an ABG yourself.
- 2nd spontaneous pneumothorax same side, no surgery, FVC 66% → **defer pending pulmonary eval**. Recent pneumothorax → first **investigate symptoms/resolution**.
- Significant thoracic scoliosis → **pulmonary consult**. Of the respiratory diagnoses, emphysema most warrants specialist evaluation.
- Pulse ox/ABG are indicated by conditions causing **airway obstruction**; spirometry by lung-disease history or symptoms such as **chest tightness/wheezing**.
- Sedating meds last **12 h**; Benadryl → defer pending PCP.
- Hypersensitivity pneumonitis → certifiable; **the driver must avoid the causative agent, including transporting it**. TB history alone → not precluded. Primary alveolar hypoventilation → unlikely ever to meet the standard.
- Anaphylaxis history → key = **successful preventive measures**.
- OSA: weigh **multiple risk factors**, never one alone; obese HTN snorer w/ witnessed apneas → **refer to sleep specialist**; sleepy obese driver, crowded airway, recent drowsy-driving crash → **disqualify + refer**; new CPAP → confirm treatment **effective, safe, stable**.
- **No required grip test; severe functional impairment = not qualified**. Isaac's → most likely precludes; MS/PD/neuropathy/muscular dystrophy → **case-by-case**; ALS/MG/metabolic muscle disease → disqualifying [legacy].
- The ME **cannot** add a lifting restriction for an employer; a driver who can't do the normal tasks is disqualified.
- Carpal tunnel or missing 2 fingers → **test grip/prehension** first. Only fixed limb deficit eligible for SPE.
- SPE: **2 yr**, renew **30 days** prior, **FMCSA** grants, no torso, demo not always required, driver **carries it**, ME **proceeds with exam**, max cert **2 yr**.
- Heavy-labor fitness is about ROM, strength, and balance; sustained attention belongs to the neurological/psychological evaluation.

Seizures, Loss of Consciousness & Diabetes

PART A – EPILEPSY, SEIZURES, LOSS OF CONSCIOUSNESS

Standard – 49 CFR 391.41(b)(8)

- Qualified if **no established history or clinical diagnosis of epilepsy** or **any other condition likely to cause loss of consciousness (LOC) or loss of ability to control a CMV**.
- Condition not presenting with seizures/LOC (or low likelihood) → evaluate under a different standard.
- Review/discuss every "yes" on MCSA-5875. If seizures reported: ask about childhood febrile, provoked, and unprovoked seizures and **how many**.

Definitions

Term	Key points
Epilepsy	Chronic functional disease; seizures occur without warning , loss of voluntary control, may cause LOC.
Single unprovoked seizure	No known cause/trigger. Not epilepsy . Recurrence most likely within 5 yrs, very low after. Risk factors: remote neuro insult (stroke), abnormal EEG, focal lesion on imaging, family history.
Second unprovoked seizure	Regardless of interval → may = history/diagnosis of epilepsy.
Provoked seizure	Known cause or clear reversible/avoidable trigger: drug reaction, high temperature, acute infection, dehydration, alcohol/drug withdrawal, acute metabolic disturbance (hypernatremia, hypoglycemia, hypomagnesemia). Normal reaction of healthy nervous system; recurrence risk = likelihood trigger recurs.
Seizure after stroke	Even a single one may = epilepsy.
Childhood febrile seizure	Age 6 mo–5 yr, healthy child, fever from infection; no adult residuals. Confirm history limited to febrile seizures → certify normally (2 yr).

Cannot be certified (advisory criteria)

1. History of epilepsy/seizure disorder (unless 10-yr criterion met or federal seizure exemption).
2. Current clinical diagnosis of epilepsy/seizure disorder.
3. Taking anti-seizure medication **to prevent seizures**.

Waiting periods (advisory criteria + Appendix A)

Situation	Wait	Max cert
Epilepsy / seizure disorder history	10 yrs seizure-free AND off anti-seizure meds	—
Single unprovoked non-epileptic seizure (treated or not)	5 yrs seizure-free AND off meds ; [legacy FMCSA tables — not in the current (Jan 2024) Appendix A]: clearance from epilepsy neurologist who understands CMV driving	1 yr
Single provoked seizure or provoked LOC	None — fully recovered, no residuals, no anti-seizure meds, recurrence/re-exposure unlikely; etiology confirmed, treatment stable	1 yr
Single LOC, unknown cause, not a seizure	6 months recommended; ME finds recurrence unlikely; no anti-seizure meds; consult treating provider	—
Bacterial meningitis / viral encephalitis, no early seizures	1 yr seizure-free, off meds	2 yrs
Bacterial meningitis with early seizures	5 yrs	2 yrs
Viral encephalitis with early seizures	10 yrs	2 yrs
Active CNS infection	Do not certify; aseptic meningitis history OK	—
Benign tumor: infratentorial meningioma, acoustic neuroma, pituitary adenoma, spinal benign, extra-axial	1 yr seizure-free after surgical removal	1 yr
Benign supratentorial tumor, spinal tumor	2 yrs seizure-free after removal	1 yr
Primary/metastatic malignant CNS tumor	Not recommended	—
TIA	1 yr	1 yr
Stroke, mild, no seizure risk	1 yr	1 yr
Stroke, cortical/subcortical	5 yrs	1 yr
Intracerebral/subarachnoid hemorrhage	1 yr (no seizure risk); 5 yrs (seizure risk)	1 yr

Situation	Wait	Max cert
Mild TBI (LOC/altered < 60 min)	No LOC: 1 month + treating-provider return-to-work note; with LOC: add neurologist eval	6 mo*
Moderate TBI (LOC 30 min–24 hr)	3 yrs ; cleared by treating MD/DO; neurologist tests motor speed, dexterity, cognition, depression	6 mo*
Severe TBI (LOC ≥ 24 hr) or penetrating brain injury	Do not certify	—

*TBI recert: q6 months in active treatment; annually for next 3 yrs after; then ME discretion. Seizure during TBI wait → apply (b)(8) seizure rules.

- Stroke/TIA/hemorrhage: need normal neurological, neuro-ophthalmological, neuropsychology evals; no residuals; neurologist clearance. **No cert if on oral anticoagulant or CNS-depressant med.**
- Seizure risk by location: **cerebellum/brainstem = not increased; cortical/subcortical = increased.**
- Stroke: recovery and recurrence mostly within 1 yr (recurrence 10–15%). TIA: 1 in 3 → stroke; ~half within 1 yr.

Federal Seizure Exemption

- Shorter wait than 5/10 yrs. ME may refer when driver has epilepsy/seizure disorder, a single non-epileptic seizure, or ME is unsure driver meets (b)(8).

History	Seizure-free	Meds
Epilepsy / seizure disorder	8 yrs on or off meds (if stopped: 8 yrs from discontinuation)	If on meds: stable 2 yrs (no change in drug, dose, frequency)
Single unprovoked seizure	4 yrs on or off meds	Stable 2 yrs if on meds
Provoked, low risk: med-induced; non-penetrating head injury LOC ≤ 30 min; brief LOC; non-recurring metabolic derangement; drug/alcohol withdrawal	Case-specific	—
Provoked, moderate-high risk: non-penetrating head injury LOC/amnesia > 30 min; penetrating head injury; intracerebral/intracranial hemorrhage; brain surgery with significant hemorrhage; brain tumor; stroke	8 yrs on or off meds	—

- ME: full exam, other standards met; **max cert 1 yr (epilepsy w/ exemption), 2 yrs (single unprovoked seizure w/ exemption)**; check "accompanied by" exemption on MCSA-5875, write "seizure exemption."
- Carrier ensures current exemption + current MEC (MCSA-5876); driver **carries both**.
- Apply: 202-366-4001; FMCSAseizureexemptions@dot.gov.

Anti-seizure meds not for seizures

- Ask **why prescribed**; evaluate under that diagnosis (lamotrigine for bipolar → (b)(9)); document e.g., "gabapentin for nerve pain" on MCSA-5875.
- Side effects: mood depression, cognitive defects, slowed reflexes, unsteadiness, **sedation**; low pain doses less likely to interfere.
- Phenobarbital (Luminal): anticonvulsant + controlled substance → also (b)(12)(ii) prescription exception; MCSA-5895 may be used.
- Tests: EEG (epilepsy, sleep disorders); MRI; lumbar puncture (tumor, infection, bleeding). Refer to PCP; document requests.

Syncope

- Symptom, not a condition.** Neurologic cause → (b)(8)/underlying standard; **cardiac cause → cardiovascular standard.**
- Pre-syncope (dizziness) vs true syncope (LOC); check med side effects (electrolyte shifts), cause, treatment. Single unknown-cause LOC → 6-month wait.

Narcolepsy / idiopathic hypersomnia

- Narcolepsy: excessive daytime sleepiness (primary sign) + REM phenomena awake (cataplexy, sleep paralysis, hypnagogic hallucinations); naps 10–20 min up to 8×/day; sleep attacks almost daily; no cure. Dx: overnight lab sleep study + Multiple Sleep Latency Test next morning.
- Both conditions: does NOT meet (b)(8), even if treated.**
- Xyrem (sodium oxybate, Schedule III) → ask underlying condition; may use Pending.

Headache, vertigo, Meniere's

- Only headaches likely to cause LOC/loss of control preclude certification.** Migraine not interfering → up to **2 yrs**. Consider frequency/severity/duration, symptoms (visual, light/noise sensitivity), med effects, treatment stable.
- Vertigo: needs vestibular, visual, proprioceptive input + CNS integration. Vertigo meds (meclizine) **sedate**. Untreated/uncontrolled → disqualify. [legacy FMCSA tables — not in the current (Jan 2024) Appendix A]: BPV / peripheral vestibulopathy → **2 months symptom-free**, max 2 yrs; the same legacy tables list Meniere's, labyrinthine fistula, nonfunctioning labyrinths as do-not-certify, but the 2024 MEH (and most courses) treat Meniere's **case-by-case** (frequency, warning?, LOC likely?, treatment stable?).
- Meniere's: inner ear, usually one ear; vertigo 20 min–hours; fluctuating hearing loss, tinnitus, ear pressure; permanent hearing loss possible.

CNS infection / tumor / TBI considerations

- Infection: confirm etiology; seizures (how many/when/meds/recurrence)? resolved? residuals? No records → request them.
- Tumor: etiology; LOC likely? treatment stable? Some benign tumors certifiable after successful surgery; imaging no tumor; deficit absent/stable.
- TBI: LOC duration? cognitive/psychosocial/sensory/motor severity? Post-TBI modafinil → assess alertness/attention, side effects (nervousness, anxiety, headache). Dementia → most likely precludes certification.

PART B – DIABETES

Regulations

- **391.41(b)(3)**: no history/diagnosis of diabetes treated with insulin **unless meets 391.46**.
- **391.46 (2018 rule)**: ITDM drivers **no longer need a federal exemption**. Qualified if otherwise meets 391.41 (or has required exemption/SPE) + evaluated by treating clinician + certification exam by ME.
- Non-insulin diabetes: **no specific standard**; must meet the 13 standards; case-by-case.

ITDM process

Item	Rule
Treating clinician (manages diabetes, prescribes insulin)	Evaluates before every cert/recert; completes & signs MCSA-5870 with date, full name, office address, phone
Timing	ME exam within 45 days ; driver gives MCSA-5870 to ME; ME retains it with MER
ME	Considers form, applies independent judgment ; may request more info
Waiting period / max cert	None / 12 months

Four 391.46 standards

1. **Stable insulin regimen** and proper control.
 2. **No severe non-proliferative or proliferative diabetic retinopathy** — disqualifying by regulation; MCSA-5870 documents it; ME may rely on it or refer for further eval. Risk: sudden vision loss (bleed/detachment); treatment impairs peripheral & night vision.
 3. **≥ preceding 3 months of electronic glucose self-monitoring records** to clinician → 12-month cert.
 4. Without 3 months of records → **cannot certify > 3 months**.
- Records: **electronic glucometer** storing all readings with **date/time**, **electronically downloadable**; printout or meter to clinician.
 - < 3 months: ME may certify **3 months** → driver collects 3 months → clinician completes new MCSA-5870 → back to ME → up to 12 months.

Certify vs do not certify (ITDM)

Certify	Do NOT certify
Meets 391.46 + 391.41; clinician attests stable plan/well controlled; valid completed MCSA-5870	Unstable regimen or poor control; < 3 months records; severe hypoglycemic episode not yet cleared under 391.46(e) (common training-course convention: "episode in last 12 months" → do not certify); any other (b) disqualifier

Severe hypoglycemic episode

- Definition: **requires assistance from others, or causes LOC, seizure, or coma**.
- **This is the regulation's mechanism (391.46(e))**: driver **prohibited from CMV operation**; report and be evaluated by clinician **as soon as practical**. To resume: clinician confirms cause addressed + stable regimen/control, completes **new MCSA-5870**; driver keeps it for next ME exam. The MCSA-5870 asks about the preceding **3 months**; the "12-month window" is a training-course convention, not the regulation.
- Hypoglycemia = greatest public-safety risk among diabetic complications.
- Non-insulin driver, severe episode 1 month ago → **disqualify**, advise treating-provider follow-up. Frequent hypoglycemia → evaluate under (b)(8).

Glucose / labs

- **No mandated glucose or HbA1c level**; ME judgment. Reference: fasting ≥ 126 mg/dL (7.0 mmol/L); non-fasting > 200 (11.1); A1c ≥ 6.5% diabetic, 5.7–6.4% at risk.
- Every exam: note sugar, blood, protein in urine; ask about diabetes/kidney disease; with consent consult provider (meds, side effects, A1c, creatinine, BUN). Glycosuria → consult treating provider. Persistent proteinuria → neuropathy/renal dysfunction, consider testing.

Non-insulin-treated diabetes

- Wait none; **max 2 yrs** if well controlled (App. A: meets all standards, no underlying condition, plan without insulin not interfering with driving).
- **MCSA-5872** = **optional** info tool (diagnosis, monitoring, stability).
- Oral agents / GLP-1 (Ozempic): no specific guideline; check side effects (dizziness, headache, fatigue).

- Ask: symptoms (polyuria, thirst/hunger, weight change, numb hands/feet)? glycosuria tested? hypoglycemia/LOC? treatment stable? compliant?
- Complications under other standards: amputation/neuropathy (b)(1),(2),(7); cardiac (b)(4); retinopathy (b)(7); HTN (b)(6).

Renal / GU

- No ESRD/dialysis standard → evaluate under underlying cause: diabetes → (b)(3); HTN/CHF → (b)(4). Ask about post-dialysis fatigue, cramps, hypotension, cognitive impairment; compliance. The MCSA-5875 physical exam includes the genito-urinary system **including hernias** (item 9).

High-yield points

- Single unprovoked seizure → **5 yrs seizure-free off meds**. Epilepsy → **10 yrs seizure-free off meds**; 2 years seizure-free is never enough without an exemption.
- 3 unprovoked seizures in 5 yrs → **disqualify outright**; deferral or referral adds nothing.
- Exemption: epilepsy **8 yrs + meds stable 2 yrs**; single unprovoked **4 yrs**. Max cert with exemption: epilepsy **1 yr**, single unprovoked **2 yrs**.
- ICU hypotension/sepsis seizure, resolved, no meds → **certify 2 yrs** (no wait). Non-seizure LOC → **6 months**. Febrile seizure at age 4 → **certify 2 yrs**.
- A **single seizure after a stroke** may by itself constitute epilepsy; provoked triggers (dehydration, childhood fever) carry no such weight.
- Cerebellar stroke, phenytoin stopped, no seizures → **assess against standard**. Cerebellar AVM bleed → **assess individually**. Ruptured AVM + hemiplegia + cognitive deficit → **disqualify**.
- TIA: **1 in 3** eventually have a stroke, about half within a year; after the 1-yr wait, max cert is **1 yr**.
- Meningitis, no records → **request hospital records**. Meningitis with early seizures → weigh frequency, etiology, and medications together.
- Anti-seizure meds **may sedate**; lamotrigine prescribed for bipolar → evaluate under **(b)(9)**; phenobarbital → the first question is **why it was prescribed**.
- Xyrem → **underlying condition**; narcolepsy fails (b)(8).
- Meclizine/BPV → **assess side effects**; BPV → ask **frequency/severity/duration**; vertigo meds → **drowsiness**; uncontrolled vertigo → **disqualify**; the symptom that matters most for driving is the **dizziness itself** (training courses key "dizziness").
- Meniere's, myotonia, Bell's palsy → **case-by-case**. Migraine → up to **2 yrs**. Benign spinal tumor → certifiable after **successful surgery**. Dementia → most likely precludes. Post-TBI modafinil → assess **alertness/attention**. TBI → memory, speech, and emotional changes all matter.
- ITDM: 391.46 sets **no A1c ceiling and no defined "safe glucose range"**; carrying rapid-acting glucose is **recommended, not required** [training]; the diabetes exemption program no longer exists; max **1 yr**; new to insulin with <3 months of records → **certify up to 3 months only if the clinician attests a stable regimen** (training courses key "disqualify" for a driver with no records at all); 1 month of records + good control → **3 months**; Levemir = insulin → **391.46**; unstable regimen disqualifies (training-course convention: an episode 18 months ago and neuropathy do not — the regulation's actual bar is 391.46(e), prohibited until cleared + new MCSA-5870).
- Severe hypoglycemia is defined by events — **needs assistance / LOC / seizure / coma** — not by a glucose number; action = **prohibited from driving, evaluate ASAP**.
- Non-insulin: max **2 yrs** (diet-controlled, A1c 7.4 → 2 yrs); Ozempic → **no specific guidelines**; symptomatic on glimepiride → **MCSA-5872**; glycosuria → **consult provider**; **proliferative retinopathy** = disqualifying by regulation; dialysis → **underlying condition**; most common diabetic eye disease = **retinopathy**.

Psychological, Drugs & Alcohol, Medications

PSYCHOLOGICAL — 49 CFR 391.41(b)(9)

Standard: no "mental, nervous, organic, or functional disease or psychiatric disorder likely to interfere with the ability to drive a CMV safely." Discretionary. **Diagnosis alone never disqualifies** — case-by-case on **function and relevant history**; the more serious the dx, the less likely to qualify.

Advisory criteria: (1) emotional/adjustment problems affect memory, reasoning, attention, judgment; often caused by physical disorders. (2) **Most critical:** ability to "appreciate, evaluate, and adequately respond to environmental strain and emotional stress." (3) Highly susceptible to **frequent emotional instability** (schizophrenia, affective psychoses, paranoia, severe anxiety, depressive neuroses) → **unlikely to meet standard**. (4) Consider medication side effects/interactions.

Assess 3 areas: the disorder itself · residual symptoms after episodes · pharmacological effects of treatment.

Mental status / assessment elements

- Review MER (MCSA-5875); **discuss every "yes."** Ask: nervous/psychiatric disorder; altered/loss of consciousness; psych meds; frequent alcohol; narcotic/habit-forming drugs.
- Observe:** dress; suspicious/evasive/hostile/threatening behavior; distraction; **flat affect**; bizarre ideas; hallucinations; dishonesty/omissions.
- Ask: hopeless/depressed (how often); memory/concentration; self-harm thoughts; suicide attempt; fights; harming others; psychiatric hospitalization; work/driving/military/legal/social history. (Tinnitus belongs to the hearing work-up, not the psych history.)
- Uncooperative driver / exam cannot be completed** → **disqualify**.

Test	Use
MMSE	Cognitive screen; risk for dementia/cognitive impairment
Neuropsychological testing	Verbal, memory, cognition, motor; aids dx of Parkinson's, TBI
Personality tests (MMPI most common)	Clarify dx, guide treatment

Document: onset & dx; med dose/frequency; limitations; potential side effects (incl. OTC); abnormal findings + effect on driving + corrective steps; testing requested. **MCSA-5895 CMV Driver Medication Form:** optional, voluntary, **driver consent**, to treating/prescribing provider.

Condition tables (Appendix A)

Condition	Waiting period	Max cert	Do NOT certify if
Mood disorders (major depression, bipolar)	6 mo symptom-free after non-psychotic major depression without suicidal behavior ; 1 yr symptom-free after severe depressive episode, suicide attempt, or manic episode	1 yr	Active psychosis; side effects interfering; prominent negative sx (compromised judgment, attentional difficulties , suicidal thoughts/behavior, overt inappropriate acts)
Schizophrenia & related psychotic disorders	6 mo symptom-free after brief reactive psychosis or schizophreniform; 1 yr symptom-free other psychotic disorders	1 yr	Diagnosis of schizophrenia ; active psychosis; negative sx; side effects
Adult ADHD	None*	1 yr	Active psychosis; side effects; negative sx
Antisocial/personality disorders	None*	1 yr	Same; PD "repeatedly manifested by overt, inappropriate acts"

May certify (all): complies with treatment; tolerates without disqualifying side effects (sedation, impaired coordination); **comprehensive eval by mental health professional who understands CMV driving**. *Not until etiology confirmed and treatment effective, safe, and stable*.*

- Bipolar:** ≥1 manic episode ± depression. Mania: irritable/expansive/elevated mood, ↓ judgment, ↑ substance abuse, ± delusions/hallucinations. Tx: lithium or anticonvulsant; antipsychotic if psychosis; antidepressants for bipolar depression.
- Major depression:** depressed/irritable mood, anhedonia, withdrawal, sleep/appetite change, fatigue, restlessness/agitation, ↓ memory/concentration, poor judgment, suicidality; psychosis in severe cases. Most recover; some relapse within **5 yr**; stressors in prior **6 mo**. Hallucinations belong to psychosis, not to a routine depressive episode.
- Schizophrenia:** most severe psychotic disorder — psychosis, negative sx, impaired cognition/judgment/attention, ↑ suicide. Related: schizophreniform, brief reactive psychosis, schizoaffective, delusional. Antipsychotic side effects worst at **initiation/dose change**.
- PTSD:** anxiety disorder after trauma; mood swings, suicidal thoughts. Exposure therapy (CBT/cognitive processing); mild-moderate treated in ~**1 yr**. Meds almost always + psychotherapy: SSRIs; **benzos** → **dependence, may worsen PTSD**; antipsychotics, mood stabilizers. Key Q: psychiatrist/psychologist eval & recommendations.
- Adult ADHD:** age-inappropriate impulsiveness/hyperactivity; mood lability, explosiveness, low frustration tolerance; comorbid antisocial/borderline PD, substance abuse. Before certifying on stimulant: **request treating-provider eval, verify adult ADHD dx, determine side effects**; check opioid/benzo interactions.
- Bipolar, ADHD, personality disorders all ↑ substance-abuse risk.**

- **Dementia:** not disqualifying by regulation; cognitive deficits (memory, planning, complex tasks, awareness) mean driver **may lack** ability; majority Alzheimer's; incurable.
- **Anxiety/panic:** no separate table; "severe anxiety" in criterion 3; benzos → prescription exception. **Eating disorders:** not addressed in source.

Psychiatric therapies

Therapy	Wait	Max cert	Key points
Antidepressants	None*	1 yr	Case-by-case. First-generation TCAs (amitriptyline/Elavil, clomipramine, amoxapine, doxepin, imipramine, nortriptyline) → more side effects (dizziness, drowsiness, ↓ alertness); Appendix A: should not certify first-gen user. Second-gen (SSRIs, norepinephrine reuptake modulators, unicyclic aminoketones) fewer effects but can still impair; tolerance to sedation develops. MAOIs listed only, no specific rule. Esketamine (Spravato) = Sched III . SSRIs not controlled → focus = history & functioning.
Antipsychotics	None*	1 yr	Typical/atypical neuroleptics; only partially correct sx. Side effects: sedation, visual disturbances (esp. night), motor dysfunction , arrhythmia, orthostatic hypotension. Also for nausea, chronic pain. Use not disqualifying itself . Lurasidone not controlled; dizziness/syncope.
Anxiolytics / sedative-hypnotics	None*	2 yr	Benzos impair; non-benzos less; barbiturates worst . Both Sched II–V → prescription exception. May certify: short-acting hypnotic (half-life < 5 h), lowest dose, < 2 weeks , or non-sedating anxiolytic . Do NOT certify sedating anxiolytic. Diazepam's half-life runs roughly 24–48 h (active metabolites longer) [clinical], so it lingers for days after the last dose; first Q = why prescribed (alcohol withdrawal ⇒ current alcoholism). Ambien: next-day impairment.
CNS stimulants (dextroamphetamine, methylphenidate, pemoline; Adderall/Vyvanse Sched II)	—	—	Narcolepsy, adult ADHD, adjunct to antidepressants; low doses ↑ vigilance/simple tasks, not complex. Controlled → request prescriber info (MCSA-5895) ; not automatically disqualifying.
ECT	6 mo symptom-free after course	—	Acute confusion, disorientation, short-term memory loss; resolves within months. Certify if waiting period done, mental-health eval, not on maintenance ECT .
Lithium	None*	—	Bipolar & depressive disorders; little evidence it impairs driving . Certify if asymptomatic and levels in therapeutic range . Side effects: drowsiness, dizziness, fatigue, fine tremor.

- Regulatory max for a history of a psychological disorder = **2 years** (391.45 — no psych-specific interval; ME decides). The legacy App-A tables recommended 1 yr for most conditions; training courses key 2 years.
- Anticonvulsant mood stabilizers: carbamazepine, lamotrigine, valproate, divalproex. **Pregabalin (Lyrica) = Schedule V**.

DRUG USE — 49 CFR 391.41(b)(12)

Part	Rule
(b)(12)(i)	Does not use any drug in 21 CFR 1308.11 Schedule I , an amphetamine , a narcotic , or any other habit-forming drug .
(b)(12)(ii) "prescription exception"	Does not use any non-Schedule I drug (21 CFR 1308) except when prescribed by a licensed medical practitioner who is licensed to prescribe controlled substances, is familiar with the driver's medical history and job duties , and has advised the driver the drug will not adversely affect safe CMV operation.

Advisory criteria: (1) Schedule I cannot be prescribed for any purpose → ME **cannot qualify**, under any circumstances. (2) ME **may** qualify amphetamine/narcotic/Schedules II–V (1308.12–1308.15) if exception met. (3) **Written communication from the prescriber** shows exception met; **MCSA-5895** = voluntary tool (driver consent). (4) ME **may request a non-DOT drug test** (e.g., signs of undisclosed use); **no SAP required** (49 CFR 40.3, 40.281); may ask for drug-use assessment by qualified professional.

- **ME makes the final decision; may disqualify even if prescriber says safe.** Based on history + exam + testing ME deems necessary.
- **No standard for non-scheduled drugs** — evaluate by **underlying condition** and side effects (vision, drowsiness, dizziness, confusion, reaction time, mood).
- **No mandatory waiting period** after disclosed illegal use; prior use doesn't preclude; ME may withhold MEC pending assessment/treatment/testing/counseling. Evasive driver, refuses non-DOT test, can't rule out current use → **disqualify**.
- Intoxication signs at exam (fresh injection marks, pinpoint pupils, unsteady gait) → **disqualify**; don't let him drive away. Dilated pupils, HR 116, clammy skin, vague history → **request non-DOT test**.
- Ask: therapeutic vs habitual; whether the driver uses drugs to change how he feels or to push past normal limits; rehab history; noncompliance risk; monitoring; interactions.

Marijuana / CBD

- Marijuana & extracts > **0.3% THC = Schedule I**. Positive test/use → **disqualified even in states legalizing recreational, medical, or religious use**; "medically prescribed" marijuana → **disqualify**.
- **CBD < 0.3% THC: not automatically disqualifying**; no federal labeling oversight → may cause positive, disqualifying test. CBD gummies → **discuss risks**. Guidance: DOT Office of Drug and Alcohol Policy; FDA.

Opioid-agonist treatment

Drug	Sched	Rule
Methadone	II	Does NOT automatically preclude certification; need prescriber info (familiar w/ history & duties) that tx won't adversely affect driving; final decision = ME. Driver a few months into a methadone program → consult the prescribing practitioner .
Suboxone (buprenorphine/naloxone)	III	Same — not automatically disqualifying; prescription exception; request prescriber info (MCSA-5895) .

DOT drug & alcohol testing — 49 CFR 382 (employer's program, not the ME's)

- **CDL/CLP holders** operating CMV on public roads: GVWR > **26,000 lb**, **16 or more passengers (incl. driver)**, or **placarded hazmat**. Includes government, owner-operators, for-hire, Mexican/Canadian-licensed drivers.
- **5 substances: marijuana, cocaine, amphetamines, opioids, PCP** (not alcohol, not nicotine).
- Required **pre-employment**; also random, **reasonable suspicion**, post-accident, return-to-duty, follow-up. **Pre-employment alcohol test = employer option, not a DOT requirement**.
- Separate from physical exam; may be **concurrent**. **Drug testing is the motor carrier's responsibility**. Oversight: DOT Office of Drug and Alcohol Policy and Compliance.
- **MER Qs 31 & 32** (illegal drug use) support (b)(12) — not meant to involve ME in Part 40/382 testing.
- **382.213**: (a) no Schedule I on safety-sensitive duty; (b) non-Schedule I only w/ prescriber advice; (c) employer w/ actual knowledge must not permit; (d) **employer may require drivers to report therapeutic drug use**.
- **392.4**: on duty, no possession/use/influence of Schedule I, amphetamines ("pep pills, bennies"), narcotics, or any incapacitating substance; carriers may not permit; practitioner-administered excepted.
- **Clearinghouse**: database of CDL/CLP drug/alcohol violations; employers, MROs, SAPs must report; since **Nov 18, 2024** SDLAs check before issuing/renewing/upgrading/transferring CDL/CLP and downgrade "prohibited" drivers until **return-to-duty** done. Doesn't directly affect MEs.
- Source gives no cutoffs, BAC thresholds, or SAP process details.

DEA schedules (Appendix B)

Sched	Definition	Examples
I	No accepted medical use; high abuse; illegal	Heroin, LSD, marijuana, peyote, MDMA, methaqualone, psilocybin, designer benzos
II	Accepted use, high abuse → " dangerous ," most strictly regulated; never OTC	Amphetamine (Adderall), methylphenidate, Vyvanse, oxycodone, hydrocodone, fentanyl, morphine, methadone, meperidine, codeine, pentobarbital, secobarbital, PCP
III	Moderate–low; psychological > physical dependence	Tylenol #3 (≤ 90 mg codeine), buprenorphine/Suboxone, anabolic steroids, testosterone, ketamine, esketamine, dronabinol, Xyrem, benzphetamine
IV	Limited dependence	Xanax, Klonopin, Valium, Ativan, temazepam, triazolam, Ambien, orexin antagonists, carisoprodol (Soma), tramadol, modafinil (Provigil), pentazocine
V	Least; antidiarrheal/antitussive/analgesic	Lomotil, Motofen, Robitussin AC, Phenergan w/ codeine, pregabalin, ezogabine

Most misused: opioids, depressants, stimulants; most common reason = **physical pain** (2016 NSDUH; >10 million/yr).

ALCOHOLISM — 49 CFR 391.41(b)(13)

- Qualified if "**no current clinical diagnosis of alcoholism**." "**Current**" = current alcoholic illness **or** condition not fully stabilized, **regardless of time involved**.
- ME may certify **prior dx in remission**; residual neurological changes evaluated under the applicable standard (alcoholic neuropathy → **(b)(7)**).
- **ME determines** current alcoholism (exam + driver info + testing). May order **non-DOT alcohol test** (excessive use disclosed or signs seen); **no SAP required**; may require professional assessment (current dx vs remission).
- "Yes" to frequent alcohol → first characterize **how much, how often, and in what pattern** the driver drinks. Signs: tremor, liver enlargement, cognitive impairment.
- Fresh out of court-ordered rehab after a DUI, still in early recovery, no professional assessment yet → **active alcoholism** → **disqualify** (may refer to SAP).
- **Disulfiram (Antabuse)** ⇒ current alcoholism → precludes certification. Other FDA alcohol meds: acamprosate, naltrexone. Valium for withdrawal ⇒ ineligible.
- Impairment via intoxication **and withdrawal**, even off-duty use.

Tool	Detail
AUDIT	WHO, 10 items ; score ≥ 8 = harmful/hazardous use [clinical]
CAGE	Cut down, Annoyed , Guilty , Eye-opener ; ≥ 2 "yes" suggests dependence [clinical — FMCSA mandates neither screen]

Alcoholism in remission (App. A): wait none (after completing counseling/treatment); **max 2 yr**; certify if no residual impairment, treatment completed, no current alcohol disorder. Do not certify: current dx; signs of illness or noncompliance w/ DOT alcohol guidelines; unstable alcohol-related condition; return-to-duty not met.

MEDICATION → CERTIFICATION (Appendix C)

Class / drug	Consideration
Narcotic analgesics (codeine, hydrocodone, oxycodone, morphine, tramadol, Demerol, hydromorphone, fentanyl)	Controlled → prescription exception
NSAIDs / non-narcotic	Generally safe; cyclobenzaprine, sumatriptan → dizziness/drowsiness
Muscle relaxants (baclofen, cyclobenzaprine, metaxalone, methocarbamol, tizanidine)	Drowsiness, dizziness, syncope, blurred vision; carisoprodol & diazepam controlled
Narcotic antitussives / antidiarrheals (Phenergan w/ codeine, Robitussin AC, Lomotil, Motofen)	Sched V → prescription exception; drowsiness, blurred vision
Antihistamines	First-gen (diphenhydramine, hydroxyzine) sedating, up to 12 h ; used for vertigo. Pseudoephedrine → drowsiness/dizziness
Sleep aids	Benzos, hypnotics, modafinil, orexin antagonists controlled; ramelteon, trazodone, doxepin not. Narcolepsy/idiopathic hypersomnia → fails (b)(8)
Anticonvulsants	To prevent seizures → cannot certify ; for pain/migraine/bipolar (gabapentin, lamotrigine) → evaluate that dx. Phenobarbital, pregabalin controlled
SSRIs / SNRIs / buspirone	Not controlled; SSRIs few major effects (nausea, drowsiness, dizziness, blurred vision)
Anticoagulants	Bleeding; monthly INR monitoring ; certify per underlying condition
Antiarrhythmics (sodium channel blockers etc.)	Qualify under cardiovascular standard (b)(4) ; cardiologist input
Antihypertensives	Beta blockers: fatigue, dizziness, depression, bradycardia, insomnia; ACE-I: hypotension/syncope; clonidine/methyldopa: drowsiness, depression
Diabetes	Insulin → 391.46 (clinician eval, glucose logs, no recent severe hypoglycemia, max 1 yr); bromocriptine drowsiness/hypoglycemia; TZDs fluid retention/HF
Anticholinergics (dicyclomine, oxybutynin)	Not controlled; ask about drowsiness, dizziness, blurred vision; worsen dementia, glaucoma, arrhythmia
Antiemetics	Dronabinol = Sched III ; promethazine, meclizine sedating
Corticosteroids	Short-term mood/sleep change; long-term mood disorders, HTN, diabetes
Dementia meds (donepezil, memantine)	Bradycardia, syncope, dizziness
Varenicline (Chantix)	Not controlled, not disqualifying; drowsiness, dizziness, ↓ focus → assess
Antibiotics / cancer meds	Acute infection doesn't preclude; cancer has no standard, not automatically disqualifying

High-yield points

- (b)(9) turns on whether a disorder is **likely to interfere** — there is no bar on history alone, on taking psychiatric medication, and no fixed year count.
- Lithium = **bipolar & depressive disorders**; monitor **therapeutic level**. CNS stimulants treat **narcolepsy**/ADHD, are scheduled, do NOT preclude. Anxiolytics treat anxiety + **insomnia**.
- Past psychosis → **case-by-case**. Chronic schizophrenia is "**unlikely to meet the standard**" — advisory language, not a regulatory disqualification; with **active psychosis**, certification is effectively out. Neither antipsychotic use nor dementia is disqualifying by regulation. The schizophreniform 6-month wait is App-A guidance, not a mandate (training courses key "no mandated wait").
- Regulatory max for a psych-disorder history = **2 years** (391.45); the legacy App-A tables recommended 1 yr for most conditions — training courses key 2 years.
- TCAs (amitriptyline, clomipramine, amoxapine) → **assess individually**; the side effect that matters most for driving is **dizziness**; amoxapine is not a controlled substance.
- An SSRI-treated driver who is visibly distracted and can't focus shows disqualifying attentional signs → **disqualify & refer**. A driver nearly a year past a suicide attempt, cleared and euthymic → the remaining question is **medication side effects**. Citalopram prescribed for GAD → what matters is **history & current functioning**.
- Adderall/Vyvanse (II), Ativan/Valium/Ambien (IV), pregabalin/Lomotil (V), Suboxone (III), methadone (II) → the move is the **prescription exception: request prescriber info / MCSA-5895**, not reflexive disqualification or a fixed 1-yr cert.

- Benzodiazepines (Xanax/Klonopin/Valium) sit in **Schedule IV**, well below the Schedule II opioids and stimulants. Tramadol = Sched IV. Naltrexone, tiotropium, sertraline, dicyclomine, varenicline, lurasidone = NOT controlled.
- Medical marijuana / MDMA → **disqualify**; CBD gummies → **discuss risks**.
- ME may order **non-DOT** tests; SAP not required; drug testing = **motor carrier's** job. Reasonable suspicion is a valid DOT trigger; anonymous tip is not. 5-panel excludes alcohol; rules = **49 CFR 382**; pre-employment alcohol test optional.
- Nicotine not prohibited; Quaalude Sched I; Schedule II = "**dangerous**." Employers **may** require reporting of therapeutic drug use.
- Alcoholism dx = **ME's** call; CAGE-positive → **non-DOT alcohol test**; sober 10 yr + neuropathy → **(b)(7)**; disulfiram → precludes.
- Most common reason for drug misuse = **physical pain** (2016 NSDUH). Hostile or threatening behavior during the exam is itself a sign of possible psychiatric disorder; ECT's hallmark after-effects are **confusion/disorientation**; neuroleptics → **visual disturbances** (esp. at night); most critical assessment = response to **environmental strain/emotional stress**.

Forms, Recordkeeping & Exam Prep

49 CFR 391.43 – Who performs the exam / rules

Rule	Detail
Who performs certification exams	ME listed on the National Registry of Certified Medical Examiners
Exception 1	Licensed ophthalmologist/optometrist may do the visual acuity, field of vision, color recognition portion only
Exception 2	Certified VA ME may examine veteran drivers only
ME must be knowledgeable about	Physical/mental demands of commercial driving; FMCSA qualification requirements; medical advisory criteria; proficient in/uses required medical protocols
Exempt intracity zone driver	Must give ME a copy of the initial medical findings that enabled intracity-zone certification
Final authority on qualification	The medical examiner (not FMCSA, PCP, or employer)

FMCSA Forms – Quick Reference

Form	Number	Purpose	Completed by	Retention / handling
Medical Examination Report (MER)	MCSA-5875	Records the full exam + determination	Section 1 driver; Section 2 ME	ME keeps ≥3 yrs from exam date ; copy to driver on request; to employer only with driver's consent ; to FMCSA within 48 hrs of request; contains private health info (HIPAA)
Medical Examiner's Certificate (MEC)	MCSA-5876	Proof of qualification	ME; signed by both ME and driver	Original to driver (paper to non-CDL drivers); copy to employer on request; ME keeps ≥3 yrs from issue date ; to FMCSA within 48 hrs; no private health info
CMV Driver Medical Examination Results	MCSA-5850	Electronic transmission of exam results to National Registry (also used to update a pending exam)	ME via National Registry account	Submit by midnight local time, next calendar day after exam
Insulin-Treated Diabetes Mellitus Assessment	MCSA-5870	Required to certify an insulin-treated driver (391.46); attests stable insulin regimen + properly controlled DM	Driver's treating clinician	To ME within 45 days of completion; file with MER
Vision Evaluation Report	MCSA-5871	Required for Alternative Vision Standard (driver fails acuity/field standard, e.g., monocular)	Ophthalmologist or optometrist	To ME within 45 days of completion; file with MER + MEC
Non-Insulin-Treated DM Assessment	MCSA-5872	Optional ; info on diagnosis, monitoring, stability of non-insulin DM	— (not specified)	File with MER
CMV Driver Medication Form	MCSA-5895	Optional ; asks why meds prescribed and whether med/underlying condition could impair driving	Treating physician on ME request	File with MER

MER (MCSA-5875) – Section 1: Driver

Item	Detail
Medical record number (top right)	ME's field; [training] may be left blank
Personal info	Name as on license , address, phone, age, DOB, license number + state; has CLP/CDL or applying
ID verification	ME verifies identity, notes photo ID type (CDL, passport)
Prior certification	Ever denied or issued for <2 years ?
Health history	Surgery (date/type); all meds incl. Rx, OTC, supplements; 32 health conditions (now/ever); space for unlisted conditions; "yes" answers must be explained (e.g., type of heart disease)
Signature	Driver signs/dates → certifies the information is accurate and complete (the form's words)

MER – Section 2: Medical Examiner

Driver Health History Review

- Discuss every **"yes" or "not sure"** response; investigate unclear answers (diagnosis, history, treatment, causes, response).
- Compare **medication list vs. history**; review all meds incl. OTC – side effects, interactions (drugs, alcohol, foods, supplements), missed-dose effects.

- Document anything the driver **omitted**; this space is for the **details of driver-reported conditions**.
- Best practice: records from treating provider (with driver consent); order tests/consults as needed.

Testing (page 2) – four required tests

Test	Recording rules
Blood pressure / pulse	Record pulse rate, height, weight, BP; if a second reading is needed, record it too
Urinalysis	Specific gravity, protein, blood, sugar; numerical values when indicated, or "trace/negative/none"; follow dipstick box instructions. Normal [clinical]: SG 1.005–1.030 (avg 1.020) ; protein/blood/sugar negative
Vision	Snellen-comparable acuity; distinguish red, green, amber ; note monocular vision, referral to ophthalmologist/optometrist, whether documentation received
Hearing	Forced whisper or audiometry

- [training] Trained assistants may perform vision, hearing, BP, urine tests – but the **ME is responsible for and attests to** all results.
- Optional tests (spirometry, grip strength, SpO2) go in **"Other testing if indicated"** (page 3); copies may be attached. ECG and drug tests are **not** required tests.

Physical Examination

- Check each body system **normal/abnormal**; discuss abnormalities in the space provided **and whether they affect safe CMV operation**.
- Memorize body-system numbering on the form: **1 = general, 2 = skin, 3 = eyes**, and so forth.
- Eyes: the MER's eye row of the physical exam centers on **pupillary equality and reactivity**.

ME Determination (federal section = exams per 391.41–391.49; must be filled out completely)

Option	How documented
Does not meet standards	Check box on MER (not MEC); write explanation; discuss reason, safety impact, steps for reconsideration
Meets standards; qualified for 2-year certification	Issue 2-yr MEC
Meets standards, periodic monitoring required	Explain why; check period: 3 months, 6 months, 1 year, or Other (specify)
Certification categories	Check all that apply: corrective lenses, hearing aid, SPE certificate, waiver/exemption
Determination pending	Need more info; no MEC issued ; up to 45 days to obtain info and decide; driver return date on or before 45-day expiration; if not updated in National Registry by then, exam invalid → re-examine ; update via MCSA-5850 ; do not use if condition interferes with safe driving
Incomplete exam	Unable to complete for any reason (e.g., driver declines to finish)
ME sign-off	Signature, name, address, phone, date; license # + state; occupation; National Registry number + ME certificate expiration date ([training] look it up in your NR profile – no longer printed on the certificate)

[training] Second opinion after disqualification: needs **employer permission**; employer may require an employer-preferred ME; second ME must get **all the same health history** and know it is a second opinion.

Medical Examiner's Certificate (MCSA-5876)

- Completed when driver is physically qualified; **both ME and driver sign**.
- Original to driver – drivers must **carry a valid MEC** when operating a CMV.
- Copy to current/prospective employer **if requested**; motor carriers **must maintain a copy**.
- Date on certificate = **date of exam**; expires **midnight on expiration date**; **no grace period**.
- **NRII rule (effective June 23, 2025)**: MEC for **CDL/CLP** drivers issued electronically – submitted to National Registry, transferred to **SDLA**; paper original still issued to **non-CDL** drivers (and on driver request).
- As of May 2026, **AK, CA, KY, LA, NH** not fully NRII-implemented → paper certificates there; FMCSA currently recommends paper to all CDL/CLP drivers. Temporary exemption: paper MEC accepted as proof for **60 days after issuance**, valid through **October 11, 2026**.

Recordkeeping & Reporting

Requirement	Rule
Retain MER	Every driver examined – ≥3 years from exam date
Retain MEC	Every driver certified – ≥3 years from issue date
Records requests	Available to FMCSA / federal, state, local enforcement within 48 hours
Transmit results	MCSA-5850 by midnight local time, next calendar day after the exam; [training] submitted through the NR account's driver-exam screens; include state-specific info for state exams
No exams in a month	Report via NR account by the last day of the month

ME certification maintenance (FMCSA audits)

- Update NR profile within **30 days** of change; [training] periodic review (e.g., quarterly) recommended. **5-yr refresher** in year 5; **10-yr recertification** training + exam in year 9 or 10.
- Provide all exam records (MER, MEC, 5870, 5871, 5895, 5872, other records used) within **48 hrs**; correct CDL/CLP applicant errors.
- Noncompliance → **Notice of Proposed Removal**: acknowledge within **30 days**, complete corrective actions within **60 days**, or be removed; wait **30 days** to request reinstatement (not guaranteed).

NRCME Certification Exam – Format & Logistics

Item	Detail
Format	Closed-book, computer-facilitated, 2 continuous hours, 120 questions, 4 options each — select the single best answer
Scored	[training] 100 scored; 20 unscored pilot items (indistinguishable)
Passing	71%
Versions	[training] Multiple test forms exist; difficulty may vary
Eligibility	Meet 49 CFR 390.103 (licensed/registered/certified by state to do physicals: MD, DO, DC, APN, PA, other authorized); complete an FMCSA-accredited training program; NR account + NR number
Scheduling	Approved testing organization listed on FMCSA site; need NR number + training certificate ; fees/locations/accommodations from testing org; remote secure-computer option may exist
Check-in	[training] Testing-org procedure — verify with yours. Typically: arrive 30 min early; bring NR number, proof of medical credential, proof of training, one government-issued photo ID ; no phones/notes/aids; misconduct → dismissal, score not reported
Results	[training] Score same day at testing center; after FMCSA verifies credentials → email with ME certificate + NR number; do not perform DOT exams until the NR credential is received
Retake	No mandated waiting period and no limit on attempts; rescheduling waits on FMCSA updating your account (can take up to 7 days); any testing org; pay the fee again

Question Levels ([training] — roughly one-third each)

Level	Tests	Original example (ours)
Recall	Facts: standards, advisory criteria, FMCSA mission, driver population, CDL definition	Which frequencies are averaged for the hearing standard? → 500, 1,000, 2,000 Hz
Application	Reach conclusions from medical info; "what should ME do next"; testing, recordkeeping	Driver's BP measures 176/108 at a first exam → one-time 3-month certificate (Stage 2)
Analysis	Clinical scenarios + FMCSA guidance + clinical judgment; refer/disqualify/other	Driver on a 2 a.m.–2 p.m. schedule, BMI 41, witnessed apneas, drowsy-driving crash last year → do not certify today; refer for sleep evaluation

[training] Two-thirds = application + analysis; expect FMCSA items beyond the curriculum (meds/side effects, normal lab values, exam skills).

Exam Content – Two Domains (FMCSA National Registry core curriculum; training required by 49 CFR 390.105)

I. Driver's Medical Information – A. Identification & history (verify ID; driver signs history; probe "yes" answers, illness/injury/surgery in last **5 years** [legacy]; meds/supplements, substances). B. Physical exam (driver gowned [training]; height/weight; eyes (b)(10); ears (b)(11); mouth/throat (b)(5); neck (b)(7); heart (b)(4),(b)(6); lungs (b)(5); abdomen; spine (b)(7); extremities (b)(1),(b)(4),(b)(7); neuro (b)(7)–(b)(9); urine; mental status (b)(9)). C. Diagnostic tests/referrals (ME may order any needed test; refer to specialist, SPE/on-road eval, incidental findings). D. Ancillary documentation (5871, 5870, SPE cert, intracity exemption, SAP results). **II. Determination & Disposition** – A. Health education counseling (contacts → carry glasses; hearing aid → keep a spare power source; DVT risk; [legacy] the defunct Federal diabetes-exemption program — replaced by 391.46 in 2018 — required glucose checks **1 h before driving and every 4 h** while driving, carrying rapid glucose, annual log submission: historical curriculum content, not current requirements). B. Risk assessment (job tasks, SPE cases, cognition, wellness stressors, sudden incapacitation). C. Certification outcome & intervals (disqualify: epilepsy/seizure history, ITDM without exemption, vision/hearing below standard without exemption, methadone (older training-material wording — current guidance says methadone does **not** automatically preclude; consult prescriber), current alcoholism, controlled substance without Rx; document reasons; certify for appropriate interval).

Study Tips

- Master FMCSA regs, advisory criteria, **ME Handbook 2024**; know hearing/seizure exemptions, SPE conditions, disqualifying meds; body-system numbers on MER. Drill with practice questions until the patterns are automatic.
- Think like an ME (**public safety first**), not a PCP. Watch absolute qualifiers ("always"/"never"); eliminate clearly wrong options before choosing the **single best** answer; skip hard items and return to them; answer every one of the 120; budget the full two hours; change an answer only when you have a concrete reason.

High-yield points

- MER retention = **3 years**. Records to FMCSA = **48 hours**.
- MCSA-5850 deadline = **midnight local time, next calendar day** after the exam.
- MEC expires **midnight on listed date, no grace period**; employer **must** keep copy; MEC contains **no** private health info; post-NRII, paper original goes to **non-CDL** drivers.
- The MER reaches an employer only with **driver consent**; it must go to FMCSA on request; it contains HIPAA-protected info.
- Driver signature certifies the information is **accurate and complete**.
- Monocular/alt vision → **MCSA-5871**, by ophthalmologist/optometrist, within **45 days**. ITDM → **MCSA-5870**, treating clinician, within **45 days**, used to **certify** insulin-treated driver.
- MCSA-5872 = **optional** extra info on non-insulin DM. MCSA-5895 = **optional**, gathers **why meds were prescribed** and whether they could impair driving.
- Four required tests: **vision, hearing, urinalysis, BP/pulse**. ECG and drug testing are not part of the DOT physical.
- Spirometry/other optional results → **"Other testing if indicated"** (page 3).
- "Determination pending" = ME **needs test results/info** obtainable within **45 days**; never for a condition that currently interferes with safe driving or for an open-ended recovery.
- Disqualification = check **"Does not meet standards" on the MER**; a MEC is issued only when the driver qualifies.
- Health History Review = **details of driver-reported conditions**; assess meds for side effects, interactions, and missed-dose effects.
- Physical Exam discussion space = **abnormalities and effect on safe CMV operation**.
- Incidental condition outside the certification decision → discuss with the driver and **refer to an appropriate provider**.
- Exempt intracity zone driver brings **initial medical findings**.
- Who decides qualification: **the ME**. Who performs exam: **NR-listed ME** (exceptions: eye doctor for vision portion; VA ME for veterans). ME must know the **physical and mental demands of commercial driving**.

2024 Handbook Cross-check

Cross-checked against FMCSA's **Medical Examiner's Handbook 2024 Edition** (120 pp) and the **2024 Handbook Update Summary**; the certification-period table below is rebuilt from those public sources and the regulations. The items in this section are where the *current* guidance differs in emphasis from the older tables still taught in most course materials — know both.

The big one: fixed waiting periods and 1-year intervals are OLD guidance

	Older guidance (Appendix A tables, 2002 Cardiovascular Advisory Panel) — still taught in most training courses	2024 ME Handbook (current)
Waiting periods (MI 2 mo, CABG 3 mo, PCI 3 wk, AF 1 mo, TBI 3 yr, stroke 1/5 yr, depression 6 mo/1 yr, etc.)	Given as specific numbers	Removed. The 2024 Handbook drops the prescriptive guidance — no waiting periods and no certification periods beyond what the regulations state; the ME makes an individual assessment using listed considerations
Cert interval for cardiac conditions (MI, CABG, PCI, HF, AF, SVT, VT, pacemaker, aneurysm, PE, stable angina, transplant, congenital HD)	1 year	"Up to 2 years" — only the regulation's 2-yr max applies
Thresholds (>6 METs, LVEF ≥40%, FEV1 <65%, PaO2 <65, 120° perimetry, neck-size cutoffs (see OSA note below), Mallampati)	Given	Mostly not stated in the 2024 MEH (LVEF bands 55–70/40–54/35–39/<35 and antihistamine "12 hours" are still in the MEH)
Numbers that ARE still fixed (regulation or 2024 MEH)	—	HTN stages (1 yr / 3 mo / 6 mo); epilepsy 10 yr; single unprovoked seizure 5 yr; LOC not-seizure 6 mo (recommended); ITDM 12 mo & 3-month records; AVS 1 yr; 45-day forms; exemption criteria (8 yr / 4 yr)

How to use this on the exam. The exam is based on the regulations, which have not changed (FR 2024-01056: the Handbook amends no regulation). Regulation-based numbers (above, right column) are safe to answer literally. For discretionary conditions, if an answer choice says "evaluate on a case-by-case basis / individual assessment / consult the treating clinician," that is the 2024-handbook answer; if the scenario reads like a classic practice-question pattern (e.g., "post-MI 3 months, satisfactory ETT, EF 48%, cleared → 1 year"), the expected answer follows the older table. When both appear, the case-by-case option is usually the "most correct."

Certification quick reference (rebuilt from the 2024 MEH and regulations)

Do not certify	Up to 1 year	Case-by-case	Up to 2 years
[reg/MEH24] Schedule I drug use · Alcoholism, current dx · Epilepsy/seizure disorder (unless 10 yr off meds / seizure exemption) · Angina, unstable/current dx ((b)(4)) · ITDM unless 391.46 met · ITDM with proliferative or severe non-proliferative retinopathy (391.46) · Acuity <20/40, field <70°, or color failure — red/green/amber (unless AVS) · Hearing fails both tests (unless exemption) · Active ICD / ICD-pacemaker · Narcolepsy · Idiopathic hypersomnia (MEH24 reading of (b)(8)) · HTN stage 3 (until ≤140/90)	[reg/MEH24] ITDM meeting 391.46 (12 mo) · Monocular under AVS (12 mo) · HTN stage 1, first instance (stage 2 = 160/100–179/109: one-time 3 mo)	[MEH24] Dementia — cognitive deficits may mean the driver lacks the ability ([training] most courses teach "do not certify") · MS · Parkinson's · ALS/motor neuron disease · Congenital myopathy · Metabolic muscle disease · Myasthenia · Peripheral neuropathy · Stroke & TIA · Cerebrovascular disease · OSA · ESRD/dialysis · Disabled ICD · Schizophrenia ("unlikely to satisfy") · [training] other conditions without regulatory numbers placed here by common training-course quick-reference cards: ADHD · Antisocial/personality disorder · Aortic regurg/stenosis · Asthma · Bipolar · CNS infection · CNS tumor · Chest wall deformity · COPD · Cor pulmonale · Cystic fibrosis · DVT · Heart murmur · Intracerebral/SAH hemorrhage · Hernia · HCM · Restrictive CM · ILD · Kidney transplant · Meniere's · MVP · Mitral regurg/stenosis · PTSD · Pulmonary valve stenosis · TBI · Vertigo	[reg/MEH24] Cardiac conditions — only the 24-month regulatory max applies: stable angina · AF · SVT · Ventricular arrhythmia · Pacemaker · Aneurysm (consider size) · Congenital heart disease · CABG · MI ([legacy] ETT 4–6 wk, repeat q2 yr) · PCI · Heart failure · Heart transplant · PE · Hearing exemption / hearing-aid users · SPE certificate · Non-insulin diabetes · Seizure, single unprovoked (5 yr off meds per App A) · LOC unprovoked, not seizure (6-mo wait recommended, MEH24) · [training] per common training-course quick-reference cards: Allergic rhinitis · Life-threatening allergy · Autonomic neuropathy · Febrile seizure (childhood) · Headache · Hypersensitivity pneumonia · Infectious respiratory disease · Intermittent claudication · Pneumothorax (spontaneous or traumatic) · Seizure, single provoked · Superficial phlebitis · Thrombotic venous disease · TB · Varicose veins

Other 2024-handbook confirmations

- **Methadone** (Schedule II) "does not automatically preclude certification" — ME needs prescriber's statement; final call is the ME's. (Older training materials listing methadone as an automatic disqualifier are superseded.)

- **Coumadin references removed**; anticoagulation judged by the underlying condition. No INR-frequency rule in the 2024 MEH.
- **Disabled ICD**: MEH adds recommendations for certification with a disabled ICD (case-by-case); active ICD still fails the CV standard.
- **Hand loss = all five fingers**; fewer fingers or any toes → (b)(2). SPE only for limb, never spine/torso. MER must record location/extent, how/when, prosthesis type.
- **Hearing exemption** application details added; **AVS**: not certifiable if deficiency unstable or insufficient adjustment time.
- **OSA**: no regulatory screening, testing, compliance, waiting period, or interval requirement; multiple risk factors → consider sleep study; retesting per treating provider. (The 2016 MRB recommendations' actual figures: neck > 17 in male / > 15.5 in female; BMI ≥ 40, or 33–40 plus ≥3 risk factors — the "neck 17/16 + BMI > 35" version is a common classroom simplification; neither set is in the MEH.)
- **Neuromuscular**: "severe functional impairment does not physically qualify"; MS and Parkinson's case-by-case; MEH gives no specific recommendations for congenital myopathies, metabolic muscle disease, motor neuron disease, myasthenia, peripheral neuropathy.
- **Stroke**: recurrence 10–15%, mostly within 1 yr; TIA → 1 in 3 eventually stroke; cortical/subcortical lesions raise seizure risk, cerebellar/brainstem do not; a single post-stroke seizure may constitute epilepsy.
- **ESRD / non-insulin diabetes / other uncovered conditions** → evaluate under the most applicable standard.
- **Privacy**: MER never to employer without driver consent, even when employer pays; driver always entitled to a copy.
- HTN Stage 2 range is 160/100–179/109 (some course materials misprint the upper diastolic bound).

Acronym Glossary

Every abbreviation used in this guide, A–Z. FMCSA form numbers are listed together at the end.

Acronym	Stands for	What it means for the DOT exam
AAA	Abdominal aortic aneurysm	Rupture risk tracks size; pulsatile mass → DQ pending imaging/CV consult
AASM	American Academy of Sleep Medicine	Source of the optional OSA screening criteria (neck size, BMI, Mallampati)
ABG	Arterial blood gas	Requested when SpO ₂ < 92%; PaO ₂ < 65 (or < 50 above 5,000 ft) or PaCO ₂ > 45 may fail the respiratory standard
ACE inhibitor	Angiotensin-converting-enzyme inhibitor	Antihypertensive class; side effects hypotension/syncope
ADD / ADHD	Attention deficit (hyperactivity) disorder	Case-by-case; stimulants are Schedule II → prescription exception, verify adult diagnosis
AF / A-fib	Atrial fibrillation	Decision hinges on underlying heart disease; anticoagulation OK if monitored
AHI	Apnea–hypopnea index	OSA severity: mild ≥ 5/h, moderate ≥ 15/h, severe > 30/h
ALS	Amyotrophic lateral sclerosis	Motor neuron disease; training courses often teach as disqualifying, MEH says case-by-case
ANSI (Z24.5-1951)	American National Standards Institute	Calibration standard for audiometry in the hearing regulation (not ISO)
AR / AS	Aortic regurgitation / aortic stenosis	Valve disease; severe AS not certifiable until repaired
AUDIT	Alcohol Use Disorders Identification Test	WHO 10-item screen; score ≥ 8 = hazardous use
AV	Atrioventricular (node/block) or aortic valve	Context-dependent: AV block → pacemaker; AV repair → 3-month wait (old guidance)
AVM	Arteriovenous malformation	Brain vascular lesion; seizure/LOC risk assessed under (b)(8)
AVS	Alternative Vision Standard (49 CFR 391.44)	Replaced the vision exemption in 2022; MCSA-5871; 1-year cert
KA	Below-knee amputation	Limb loss → needs SPE certificate; with SPE may certify 2 years
BMI	Body mass index	OSA risk factor (> 35 in AASM screen)
BP	Blood pressure	Staged 1/2/3 → 1 yr / one-time 3 mo / DQ until ≤ 140/90
BPPV / BPV	Benign paroxysmal positional vertigo	Old guidance: 2 months symptom-free; meclizine sedates
BUN	Blood urea nitrogen	Renal function lab the ME may request for diabetics
CABG	Coronary artery bypass graft	"Remedial" — does not preclude certification; sternal healing ~3 months
CAGE	Cut down / Annoyed / Guilty / Eye-opener	4-question alcohol screen; ≥ 2 "yes" suggests dependence
CBD	Cannabidiol	Legal if < 0.3% THC, but unregulated labeling can cause a positive (disqualifying) test
CBT	Cognitive behavioral therapy	PTSD/depression treatment; no certification impact itself
CDL / CLP	Commercial driver's license / commercial learner's permit	≥ 26,001 lb, 16+ passengers, or placarded hazmat; subject to DOT drug testing and SDLA reporting
CF	Cystic fibrosis	Case-by-case; key = can driver get therapy while working
CFR	Code of Federal Regulations	49 CFR 391.41–391.49 = the driver medical regulations
CHD	Coronary heart disease	LV function is the major prognostic factor; ETT/LVEF work-up
CHF / HF	Congestive heart failure / heart failure	Symptomatic HF or LVEF < 40% → do not certify (old guidance)
CM	Cardiomyopathy (HCM hypertrophic, RCM restrictive)	Case-by-case; cardiology input
CME	Continuing medical education	Not required for NR listing (only 5-yr refresher, 10-yr recert)
CMV	Commercial motor vehicle	≥ 10,001 lb, > 8 pax for hire, > 15 pax not for hire, or placarded hazmat
CNS	Central nervous system	CNS infection/tumor → seizure risk, waiting periods under (b)(8)
COPD	Chronic obstructive pulmonary disease	DQ if hypoxia at rest, chronic respiratory failure, cough syncope, or needs O ₂
CPAP	Continuous positive airway pressure	OSA treatment; FMCSA sets no compliance rule — "effective, safe, stable"
CV	Cardiovascular	Standard (b)(4)
DEA	Drug Enforcement Administration	Maintains the drug schedules (21 CFR 1308)
DM / ITDM / NITDM	Diabetes mellitus / insulin-treated / non-insulin-treated	ITDM → 391.46, MCSA-5870, max 1 yr; NITDM → up to 2 yrs, MCSA-5872 optional
DOAC	Direct oral anticoagulant	Apixaban, dabigatran, etc.; no INR monitoring; judge the underlying condition
DOT	Department of Transportation	Parent of FMCSA; "DOT physical" = the ME exam

Acronym	Stands for	What it means for the DOT exam
DQ	Disqualify / disqualified	Shorthand used in this sheet
DTR	Deep tendon reflex	Part of the neurological exam on the MER
DVT	Deep vein thrombosis	PE source; certify once treated and anticoagulation monitored
ECG / EKG	Electrocardiogram	Not a required test; ME may order; interpreted by cardiologist
ECT	Electroconvulsive therapy	Old guidance: 6 months symptom-free, not on maintenance ECT
EEG	Electroencephalogram	Seizure/sleep-disorder test; abnormal EEG raises recurrence risk
EF / LVEF	(Left ventricular) ejection fraction	55–70 normal; 40–54 slightly low; 35–39 moderately; < 35 severely low; ≥ 40 to certify (old guidance)
ESRD	End-stage renal disease	No standard of its own → evaluate under the causative condition (DM, CV)
ETT	Exercise tolerance (treadmill) test	Old guidance: > 6 METs, ≥ 85% predicted HR, SBP rise ≥ 20, no ischemia; post-MI at 4–6 wks, repeat q2 yrs
FDA	Food and Drug Administration	CBD labeling oversight (none); drug approvals
FEV1 / FVC	Forced expiratory volume in 1 s / forced vital capacity	Spirometry; asthma FEV1 < 65% → no cert (old guidance)
FMCSA	Federal Motor Carrier Safety Administration	Writes/enforces the regs; runs the National Registry; grants exemptions and SPEs
FMCSR	Federal Motor Carrier Safety Regulations	49 CFR Parts 300–399
GAD	Generalized anxiety disorder	Case-by-case; benzodiazepines need prescription exception
GCWR / GVWR	Gross combination / gross vehicle weight rating	Weight basis for CMV (10,001) and CDL (26,001) definitions
GLP-1	Glucagon-like peptide-1 agonist (e.g., semaglutide/Ozempic)	Non-insulin diabetes drug; no specific FMCSA guideline
HbA1c / A1c	Glycated hemoglobin	Diabetes control marker; no FMCSA cutoff ; ME may request from treating provider
HCM	Hypertrophic cardiomyopathy	Case-by-case; sudden death may be first sign
HHS / OCR	Health & Human Services / Office for Civil Rights	Enforces HIPAA
HIPAA	Health Insurance Portability and Accountability Act	No PHI release without consent; MER not to employer without driver consent
HOS	Hours of service (49 CFR 395)	11 h driving / 14 h duty / 60 per 7 or 70 per 8 days
HR	Heart rate	Recorded with BP; ETT target ≥ 85% predicted max
HTN	Hypertension	Standard (b)(6); staged certification
ICD	Implantable cardioverter-defibrillator	Active ICD → not certifiable; disabled ICD → case-by-case (2024 MEH)
ICH / SAH	Intracerebral / subarachnoid hemorrhage	Old guidance: 1-yr wait (5 yrs if seizure risk)
ILD	Interstitial lung disease	Case-by-case; PFTs if breathless with driving tasks
INR	International normalized ratio	Warfarin monitoring; old guidance wants results at exam and ≥ monthly checks
IOP	Intraocular pressure	Normal 10–20 mmHg; glaucoma
ISO	International Organization for Standardization	Audiometer calibration; convert to ANSI by subtracting 14 / 10 / 8.5 dB
LOC	Loss of consciousness	Standard (b)(8); unknown-cause non-seizure LOC → 6-month wait recommended
LV / LVEDD / LVESD	Left ventricle / end-diastolic / end-systolic dimension	Valve-disease certification cutoffs (e.g., AR: LVEDD ≤ 60, LVESD ≤ 50 mm)
MAOI	Monoamine oxidase inhibitor	Older antidepressant class; side-effect concerns
MCSA	Motor Carrier Safety Administration (form prefix)	See form list below
ME	Medical examiner	NR-listed clinician who performs and certifies the exam; makes the final decision
MEC	Medical Examiner's Certificate (MCSA-5876)	Proof of qualification; driver carries it; keep copy 3 yrs
MEH	Medical Examiner's Handbook	FMCSA guidance document; 2024 edition current
MEP / MRB	Medical Expert Panel / Medical Review Board	FMCSA advisory bodies whose reports become guidance
MER	Medical Examination Report (MCSA-5875)	The exam record; keep 3 yrs; contains PHI
METs	Metabolic equivalents	Exercise capacity; > 6 METs = heavy work of commercial driving
MI	Myocardial infarction	Old guidance: 2-month wait, ETT, LVEF ≥ 40%, 1-yr cert
MMPI / MMSE	Minnesota Multiphasic Personality Inventory / Mini-Mental State Exam	Personality test / cognitive screen

Acronym	Stands for	What it means for the DOT exam
MR / MS (valve)	Mitral regurgitation / mitral stenosis	Symptom development is the key concern
MRI	Magnetic resonance imaging	Brain imaging for seizure/tumor work-up
MRO	Medical review officer	Physician who reviews employer DOT drug-test results — not the ME's role
MS	Multiple sclerosis	Case-by-case; progressive → not SPE-eligible
MSLT	Multiple sleep latency test	Narcolepsy diagnosis (after overnight sleep study)
MVP	Mitral valve prolapse	Usually benign; DQ if symptoms, arrhythmia, emboli
NOPR	Notice of Proposed Removal	FMCSA audit action: acknowledge in 30 days, correct in 60
NPI	National Provider Identifier	Not a field on the MER
NR / NRCME	National Registry (of Certified Medical Examiners)	FMCSA database; you get a 10-digit NR number; results uploaded by midnight next day
NRII	National Registry II	June 23, 2025 rule: electronic MEC transmission NR → SDLA for CDL/CLP drivers
NSVT / VT / VF	Non-sustained / sustained ventricular tachycardia / fibrillation	Sustained VT ≥ 30 s or LVEF < 40% with arrhythmia → DQ (old guidance)
NTSB	National Transportation Safety Board	Investigated the 1999 Mother's Day crash that led to the NRCME
OSA	Obstructive sleep apnea	Multiple risk factors → consider sleep study; treated OSA doesn't preclude
OTC	Over the counter	Must be listed on the MER; sedating antihistamines last 12 h
PaO2 / PaCO2	Arterial partial pressure of O2 / CO2	ABG thresholds (65 / 45 mmHg)
PAD / PVD	Peripheral arterial / vascular disease	Claudication; pain at rest → DQ; 3-month post-surgery wait (old)
PCI	Percutaneous coronary intervention	Stent; 3-week wait after elective uncomplicated PCI (old guidance)
PCP	Primary care provider	"Treating provider"; consult with driver consent
PE	Pulmonary embolism	3 months PE-free on anticoagulation (old guidance)
PFT	Pulmonary function test	Indicated for lung-disease history, symptoms, smoker ≥ 35 y
PHI	Protected health information	HIPAA-covered; MER contents
POTS	Postural orthostatic tachycardia syndrome	Autonomic; DQ if resting tachycardia/orthostatic BP
PTSD	Post-traumatic stress disorder	Case-by-case; benzos may worsen; psychiatrist input
ROM	Range of motion	Neck (mirrors), hips/knees > 90° (cab entry); no required values
SAP	Substance abuse professional	Evaluates drivers after DOT drug/alcohol violations; ME may refer but SAP eval not required for ME decision
SBP	Systolic blood pressure	ETT: should rise ≥ 20 mmHg
SDLA	State driver licensing agency	Holds CDL data and med-cert status; receives MEC (electronically under NRII)
SG	Specific gravity (urine)	Normal 1.005–1.030 (avg 1.020)
SGLT2 / DPP-4	Sodium-glucose cotransporter-2 / dipeptidyl peptidase-4 inhibitors	Oral diabetes drug classes; evaluate side effects
SOB / DOE	Shortness of breath / dyspnea on exertion	Symptoms triggering respiratory or cardiac work-up
SPE	Skill Performance Evaluation certificate (49 CFR 391.49)	FMCSA-issued for fixed limb deficits; valid 2 yrs; renew 30 days before; driver carries with MEC
SpO2	Peripheral oxygen saturation (pulse ox)	Normal 95–100%; < 92% → ABG
SSRI / SNRI	Selective serotonin / serotonin-norepinephrine reuptake inhibitor	Not controlled; assess side effects and functioning
SVT	Supraventricular tachycardia	Usually not syncope-causing; ablation often curative; 1-month wait (old)
TB	Tuberculosis	History doesn't preclude once treated/non-infectious
TBI	Traumatic brain injury	Old guidance: mild 1 mo, moderate 3 yrs, severe (LOC ≥ 24 h) no; MEH: case-by-case
TCA	Tricyclic antidepressant	First-generation; more sedation/dizziness; assess individually
THC	Tetrahydrocannabinol	Marijuana's active drug; > 0.3% = Schedule I
TIA	Transient ischemic attack	1 in 3 → stroke; old guidance 1-yr wait, 1-yr cert
TM	Tympanic membrane	Ear exam; scarred TM with 5-ft whisper other ear → certify
TWIC	Transportation Worker Identification Credential	Acceptable photo ID at the testing center
UA	Urinalysis	Required test: SG, protein, blood, sugar (dipstick)

Acronym	Stands for	What it means for the DOT exam
VA	Veterans Affairs	VA-certified MEs may examine veteran drivers only
WPW	Wolff-Parkinson-White syndrome	Accessory-pathway SVT; certifiable after treatment/clearance (old guidance)

FMCSA forms (MCSA numbers)

Form	Name	Who fills it / why
MCSA-5850	CMV Driver Medical Examination Results	ME uploads to National Registry by midnight of the next calendar day; also used to update "determination pending"
MCSA-5870	Insulin-Treated Diabetes Mellitus Assessment	Treating clinician; required for ITDM; ME exam within 45 days
MCSA-5871	Vision Evaluation Report	Ophthalmologist/optometrist; required for the Alternative Vision Standard; within 45 days
MCSA-5872	Non-Insulin-Treated Diabetes Assessment	Optional information form
MCSA-5875	Medical Examination Report (MER)	Driver (Section 1) + ME (Section 2); keep 3 years
MCSA-5876	Medical Examiner's Certificate (MEC)	ME and driver sign; driver carries; keep copy 3 years
MCSA-5895	CMV Driver Medication Form	Optional; prescriber explains why a medication was prescribed and whether it impairs driving